WORKING
WITH DOCTORS
WORKING
FOR PATIENTS



General Medical Council

Our annual report: 2015

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A word from our Chair



The General Medical Council plays a vital role in helping to protect patients and improve the standards of medical practice in the UK. Safe, high-quality patient care requires a high-quality and professional medical workforce and in the UK we are fortunate to have some of the best doctors in the world. During my first year as Chair, I have enjoyed meeting with hundreds of doctors across the country. It's clear from the issues they raised that healthcare is changing at a scale and pace we have not seen before. As an organisation our first concern is for the safety of patients. It is essential that we meet current challenges head on, both for the sake of these patients and for the doctors working hard to provide their care.

Stepping up support for medical students and doctors in training

In recent months, the issues surrounding the first doctors' strikes in three decades have been at the forefront of our minds. While an agreement has now been reached, subject to a ballot of BMA junior members, it is clear to me as someone who continues to practise in the NHS that doctors in training feel less valued and less supported than when I was a newly qualified doctor, more than 30 years ago. We need to increase our efforts to improve the experience of doctors in training wherever they work. We introduced new standards in January 2016 that will be vital in making sure that doctors in training are given the appropriate support.

Improving how we deal with complaints about doctors

We have continued with our major change programme to reform how we deal with complaints about doctors. I'm delighted that we have been able to reduce some of the time and stress involved with investigating a doctor's actions. In 2015 we closed nine out of ten cases within six months and we are doing more than ever to make sure that issues are dealt with locally where possible. I hope that the changes to the law we are seeking will allow further radical reforms to come to fruition in the near future.

A more transparent medical register

This year we will publish plans to make the medical register much more transparent and useful.

Throughout all of this change, we must remember that the values that unite us – commitment, professionalism and compassion – continue to define medicine in the UK. This report sets out how our work in 2015 helped deliver this.

Professor Terence Stephenson, May 2016

A word from our Chief Executive



We are very clear as an organisation that we are here for patients – our purpose is to do everything we can to make sure that the services provided by doctors are safe and effective, that the care given is compassionate and that whenever possible patients are able to make their own decisions about the care and treatment they receive. We are also trying to do everything we can to help improve medical practice – working with patients, doctors, employers and others involved in healthcare delivery. This has been an extraordinarily difficult year for the medical profession in the UK and there is no doubt that this has and continues to have an impact on the care they provide for patients. We must be aware of the context in which doctors work and continue to do what we can to support them to provide the highest-quality care possible.

Two major challenges stand out. The first is the continued pressure on services – universally acknowledged – throughout the UK. It is an environment where mistakes are more likely to be made, where morale can be affected and where professional relationships can be strained. It is also an environment in which individual doctors can feel less able to cope both personally and professionally.

In response, we have sought to understand this context. The impact on education and training has been evident – during 2015 the number of hospital departments under our enhanced monitoring regime increased from 29 to 53. Feedback from the 16,733 doctors who have engaged with us through our Regional Liaison Service and devolved offices has been extremely positive in helping doctors reflect on and consider changing their practice, but it too has underlined the pressure many feel they are under.

The second challenge is that, for the first time, a group of doctors has taken industrial action, which includes withdrawing from the provision of even emergency treatment for their patients. Whatever the merits of their case or the opposing case made by ministers, this has been damaging for patients and indeed for the doctors themselves.

Our response has been first to recognise that we have no role in such contractual disputes, and secondly to recognise our prime duty to protect the safety of patients. Our advice to the doctors taking or contemplating industrial action has therefore, of necessity, been to acknowledge that Parliament has not fettered their right to strike, but also to remind

them of their duties under the *Medical Act* and the guidance we issue that flows from that.

We have been conscious too that much of our business as usual has also been affected by the current climate. Since the introduction of revalidation in 2012, we have now renewed the licences of more than 137,000 doctors.

Overall, this has been a significant success. The system itself works well for the vast majority of doctors, more than two million patients have given feedback on their doctors for the first time, and senior doctors responsible for running the system locally have reported positively on how revalidation is working. At the same time though we need to be careful that every aspect of the process adds value and we do not impose unnecessary work on frontline practitioners. We can see real gains for patients, doctors and employers but as we take this forward we have commissioned a review into what has worked and what needs to be improved.

Despite these successes as an organisation, we must take heed of the strength of feeling amongst the profession and continue to work with others for much needed change.

Understanding doctors' experiences

We have long recognised the need for greater understanding of doctors' experiences.

As part of our response to the review of doctors who take their own lives during our fitness to practise procedures, we are conducting a fundamental review of our approach to health cases. In December 2015, we appointed Professor Louis Appleby to advise us as we develop proposals to improve how we deal with health cases.

We also continued to take forward the recommendations from Sir Anthony Hooper's review of whistleblowing cases. We are currently focusing on how to improve the culture and handling of concerns.

Working for radical change

We have been campaigning for some time for change to the laws that shape our work. We want to make our processes more effective and straightforward for patients and for doctors.

At the end of 2015, the UK Government announced that it won't be taking forward the Law Commissions' Bill that was originally proposed. Instead, it intends to take a new, and potentially more radical, look at the future shape of medical regulation. We will engage with the four governments of the UK, with other regulators and with our key interests to help shape these proposals, which are so desperately needed.

Patient safety always first

In order to protect patients effectively we are increasingly sharing information, data, insight and best practice with others.

Our own data, intelligence and systems are now at the heart of our efforts to identify and reduce risks to patient safety. We are continually driving improvements to how we work, which make a real difference to our understanding of the world we regulate – our Data Strategy is already transforming how we work and how we work with others and we have the prospect of greater transparency and new ways of using our data and our intelligence for the benefit of patients and the profession.

Niall Dickson, May 2016

ABOUT **US**

Our role

We are an independent organisation. We help protect patients and improve medical education and practice across the UK by delivering our core functions.

- We set the standards that doctors need to follow, and make sure that they continue to meet these standards throughout their careers.
- We decide which doctors are qualified to work here and we oversee UK medical education and training.
- We take action to prevent a doctor from putting the safety of patients, or the public's confidence in doctors, at risk.

Healthcare is a complex and changing environment that continues to face major pressures. Healthcare organisations and the doctors, nurses and other professionals who work for them are being asked to do more with the same or fewer resources. Medical careers and roles are becoming increasingly complex.

We have to make sure our approach remains appropriate and is tailored to meet the different challenges of each UK country. We achieve this by having a dedicated presence in all four countries with offices in Wales, Scotland and Northern Ireland* and in England offices in London and Manchester. We also continue to develop our regional presence through our Regional Liaison Service† and our Employer Liaison Service.‡

We've changed a great deal in the past 158 years and we continue to adapt to make sure we maintain our relevance to doctors and patients. Our *Corporate strategy 2014–17*§ sets out how we will continue to change to meet an increasingly challenging external environment. This report sets out how we've made great steps in delivering against this strategy in 2015.

Each year we also produce our report, *The state of medical education and practice in the UK*.** It sets out much of what we know about the medical profession and the challenges it faces, drawing on our own data and, where appropriate, data from other sources. Our aim in publishing this information is to promote discussion and debate on issues and trends that need attention or further analysis, to improve standards of medical practice.

For 2015, we also produced an impact report that gives further insight into some of the ways we made a difference to protecting patients and improving standards of medical education and practice. This is available on our website, and includes statistics, case studies and illustrations of our work in action. Our Impact Report will be published alongside this report.††

* See www.gmc-uk.org/devolved.
† See www.gmc-uk.org/rls.
‡ See www.gmc-uk.org/concerns/11956.asp.
§ See www.gmc-uk.org/publications/corporate_publications.asp.
** See www.gmc-uk.org/somep.
†† See www.gmc-uk.org/publications/corporate_publications.asp.

Our trustees and accountability

We are independent of UK Government and the medical profession and accountable to Parliament. Our powers are given to us by Parliament through the *Medical Act 1983*.*

We work with the Professional Standards Authority, an independent body, which is accountable to Parliament and scrutinises and oversees our work, together with other health and social care professional regulatory bodies in the UK.

We are registered with the Charity Commission for England and Wales (1089278), and with the Office of the Scottish Charity Regulator (SC037750).

Council is our governing body. It comprises 12 members, six of whom are medical members and six of whom are lay members. Our Council members are also the trustees of the organisation. Council contains members from the four countries of the UK.

The trustees are responsible for making sure that we fulfil our charitable purpose and statutory purpose of protecting, promoting and maintaining the health, safety and well-being of the public by ensuring proper standards in the practice of medicine. Our core regulatory functions all support this purpose.

The trustees present this report and financial statements for the year ending 31 December 2015. In preparing this report, the trustees have complied with the *Charities Act 2011*, Charity Commission guidance on public benefit in exercising their powers and duties, and applicable accounting standards. The statements are in the format required by the Charities SORP (FRS 102) *Accounting and Reporting by Charities: Statement of Recommended Practice*.† The trustees also considered and confirmed that the aims and objectives, as set out in our *Corporate strategy 2014–17*,‡ are for public benefit and support our charitable purpose.

Our trustees have a duty to act impartially and objectively, and take steps to avoid any conflict of interest arising as a result of their membership of, or association with, other organisations or individuals. As trustees, members have a duty to avoid putting themselves in a position where their personal interests conflict with their duty to act in the interests of the charity, unless authorised to do so. To make this fully transparent, we publish a register of members' interests on our website.§

You can find out more about our governance and management structure on page 64.

* See www.gmc-uk.org/about/legislation/uk_primary_legislation.asp.
† See www.charitiessorp.org/media/619101/frs102_complete.pdf.
‡ See www.gmc-uk.org/publications/corporate_publications.asp.
§ See www.gmc-uk.org/about/council/register.asp.

Our achievements and performance

Some key achievements in 2015

In 2015, we continued work towards fulfilling our *Corporate strategy 2014–17*. We made significant progress on a number of projects, including the following.

- **We made good use of our data and intelligence* to protect patients.** This includes the fifth version of our flagship report, *The state of medical education and practice in the UK*. The report uses our intelligence and data to summarise key issues affecting the medical profession. In 2015, for the first time, we published in full the report's reference data, which contains the insights used to produce the report. This is one of the largest publications of data of any health regulator. You can find more information about the report on page 31.

- **We successfully rolled out further meetings with doctors and patients across the UK.** We understand that fitness to practise hearings can be stressful for witnesses and doctors. By sharing information at an earlier stage through the meeting process, we can avoid unnecessary hearings, speed up the resolution of cases and reduce the stress for all involved. You can find more information about meetings with doctors and patients on page 27.

- **We revalidated more doctors than ever before.** In 2015, we revalidated† over 57,500 doctors. The revalidation process let them demonstrate that their skills were up to date, that they were fit to practise in their chosen field and that they were able to provide a good level of care. Over 137,000 doctors have revalidated since the programme launched in December 2012. You can find more information about revalidation on pages 21-23.

- **We listened to others when we developed new guidance.** From early on in the process, we worked extensively with partner organisations, such as medical schools and deaneries, to develop our new standards for medical education and training, *Promoting excellence*. The benefit of early engagement was reflected in the strong support for the new standards. You can find more information about the new standards on page 16.

- **We oversaw the Medical Practitioners Tribunal Service (MPTS) becoming a statutory body and gained the right of appeal for MPTS decisions.** We achieved this through amendments to the *Medical Act 1983* and through supporting operational changes we made to improve our processes. You can find more information on these significant changes on page 25.

* We define 'intelligence' as information that we have gathered and assessed to help support and improve our decision making and performance.

† Revalidation is the process by which all licensed doctors are required to demonstrate on a regular basis that they are up to date and fit to practise in their chosen field and able to provide a good level of care. See www.gmc-uk.org/doctors/revalidation/9627.asp.

Challenges and lessons in 2015

Healthcare is a challenging environment. We continue to listen to the needs of patients, doctors, doctors in training, members of the public and a wide range of our other audiences and partners to learn how we can be a more effective regulator.

These are some ways we have continued to learn in 2015.

- **We started to implement a change programme** to make sure the organisation can deliver a model of regulation that is fit for purpose in the face of a dynamic healthcare and legislative environment. This programme will rebalance our organisation to make sure our service continues to ensure we remain financially sustainable in the future and can demonstrate strong stewardship of our resources. You can find more information about this change programme on page 39.

- **We started work to address the recommendations of an independent review** of instances where doctors have taken their lives while in our fitness to practise procedures. The review was published at the end of 2014 and led by Sarndrah Horsfall, an independent consultant. You can find more information about this important work on page 26.

- **We developed new ways of supporting whistleblowers** following an independent review by Sir Anthony Hooper into how we deal with doctors who have been referred to us, and who may have raised concerns in the public interest. You can find more information on pages 30-31.

- **We continue to evaluate the impact of revalidation as we approach the end of its first cycle.*** We have commissioned wide-ranging, independent research to evaluate its effect in the form of a three-year review by UMbRELLA† (UK Medical Revalidation Evaluation coLLAboration) and an additional year long review by Sir Keith Pearson. You can find more information about these projects on page 22.

We will need to continue to work hard to overcome some ongoing challenges that we, and others in the healthcare environment, face. These include:

- **divergence in the UK political and healthcare landscape.** In 2015, we saw further divergence and developments in the politics, healthcare delivery models and consequent practice of medicine across the UK. You can find more information on page 36.

- **legislative change in the UK and Europe.** In 2015, the UK Government said it would look at the future shape of professional regulation and the possibility of producing new legislation. You can find more information on page 36.

* December 2012 to March 2016. See www.gmc-uk.org/doctors/revalidation/12382.asp.

† See www.umbrella-revalidation.org.uk.

- **an alert mechanism with our European counterparts.** Introduced in 2015, it commits all European medical regulators to inform each other, within three working days, when a doctor's practice has been restricted or they have fraudulently attempted to access the medical register. This may present a challenge for us around the volumes of alerts we send to and receive from European regulators. We will monitor the volumes closely throughout 2016. You can find more information on page 39.

You can also find more information on our current and future challenges in the risk management section on page 61-63.

Operational activity in 2015

In carrying out our statutory obligations and core functions we saw changes in activity in 2015 compared with 2014:

- **Our register has seen 2% growth** from 267,100 doctors at the end of 2014 to 273,700 doctors at the end of 2015. The number of doctors on the GP Register has also increased by 2% and the number of doctors on the Specialist Register* by 4%. We have also seen a 3% decrease in the percentage of registered doctors who are licensed† – from 89% of our register at the end of 2014 to 86% in 2015 (although the actual number of licensed doctors has only decreased by 1% from 236,800 in 2014 to 234,600 in 2015). We believe this is because we have continued to emphasise that doctors should only hold a licence when they need one and that registration without a licence is a positive choice for doctors who wish to remain registered but who are no longer in clinical practice in the UK. For more information about our registration process please go to page 44.

- **We saw a 2.1% decrease in the number of fitness to practise enquiries we received from 9,624 in 2014 to 9,418 in 2015.** Of the fitness to practise enquiries we received we saw a 1.8% increase in the number of enquiries we closed without further investigation from 6,310 in 2014 to 6,426 in 2015. At the same time, we saw a 13% decrease in the number of enquiries we promoted for full investigation from 2,730 in 2014 to 2,381 in 2015. This is primarily due to the introduction of a new enquiries process that allows more initial investigation work to be undertaken before we make the decision to close or promote an enquiry. For more information on our fitness to practise enquiries and outcomes please go to page 46.

* The Specialist and GP Registers are registers of specialists and general practitioners eligible to work in the UK health service. The List of Registered Medical Practitioners shows if a doctor is on the Specialist or GP Register, or both. See www.gmc-uk.org/doctors/medical_register.asp.

† Doctors who want to practise medicine in the UK legally need a licence. The licence gives a doctor the legal authority to undertake certain activities in the UK. See www.gmc-uk.org/doctors/licensing.asp.

- **We saw a 13% increase in the number of fitness to practise referrals to the MPTS**, from 303 in 2014 to 343* in 2015. We believe this is due to the progression of a large number of cases against specific doctors, and efforts by the investigation teams to resolve older fitness to practise casework. For more information on MPTS tribunals please go to page 48.

- **We increased the number of enhanced monitoring quality assurance visits† we undertook by 83%**, with 29 visits in 2014 and 53 in 2015. These are reviews of local education providers where we are concerned about progress on identified concerns. The rise in enhanced monitoring is likely to be driven by multiple factors including the focus we increasingly place on this work, the move to a regional working structure that has given us stronger, local, relationships, and increased pressures on the NHS and service delivery in general. We are streamlining our regional reviews‡ to reallocate resources to respond to the enhanced monitoring increase. You can find out more about enhanced monitoring on page 50.

- **We saw email enquiries and other general correspondence dealt with by our contact centre increase by 16%**, from 89,713 in 2014 to 104,178 in 2015. In recent years, emailing has become the preferred channel of communication for many doctors. However, we also experienced a significant increase in requests for Certificates of Current Professional Status.§ For more information on our enquiries please go to page 51.

* The figure covers all referrals from the GMC to the MPTS, which will include outcomes of some fitness to practise investigations and referrals from the Registrar as part of the registration process.

† We carry out visits to medical schools, deaneries, local education and training boards (LETBs), and local education providers. See www.gmc-uk.org/education/medical_school_and_deanery_visits.asp.

‡ A regional review involves visiting the medical school or school, the organisations responsible for postgraduate training, and some NHS trusts or boards which are responsible for delivering training in a region. We conduct regional reviews in England and national reviews in Northern Ireland, Wales and Scotland. See www.gmc-uk.org/education/13707.asp.

§ The Certificate of Current Professional Status (CCPS) provides a range of information about the current status of a healthcare professional who is seeking registration with a regulator in another country. The CCPS is the name used within Europe, and we also have a Certificate of Good Standing, which is the name used throughout the rest of the world. See www.gmc-uk.org/doctors/information_for_doctors/28500.asp.

> “Useful from start to end. I gained confidence and my awareness increased.”
>
> “Helped me get a better understanding of the doctor/patient relationship and how to work collaboratively with colleagues.”

Doctors’ feedback after attending one of our sessions

2015 in numbers

924
patients and members of the public we spoke with directly about our standards for good practice



95%
of doctors and medical students surveyed said they would reflect on their practice as a result of attending an engagement event run by one of our offices in Northern Ireland, Scotland or Wales.

16,733
doctors we spoke with directly about our standards for good practice

76
visits to check the quality of education and training

8,583
concerns about doctors reviewed

2014 — 2015

18,493
students and educators we spoke with directly about our standards for good practice

53,126
of doctors in training responded to our 2015 national training survey





96%
of doctors and medical students surveyed said they would reflect on their practice as a result of attending one of our Regional Liaison Service engagement events.*

76%
of doctors and medical students surveyed said they would change their practice as a result of attending a Regional Liaison Service engagement event.



* Devolved Office and Regional Liaison Service figures disaggregated as Devolved Office Liaison advisers came into post in Q3 2015. Devolved Office figures therefore relate to Q2-Q4 2015.

† Represents total visits and not total visit days, ie 1 visit may have had 10 visit days.

66% of doctors and medical students surveyed said they would change their practice as a result of attending an engagement event run by one of our offices in Northern Ireland, Scotland or Wales.

57,602
doctors revalidated

2,381
full investigations launched into doctors' fitness to practise medicine

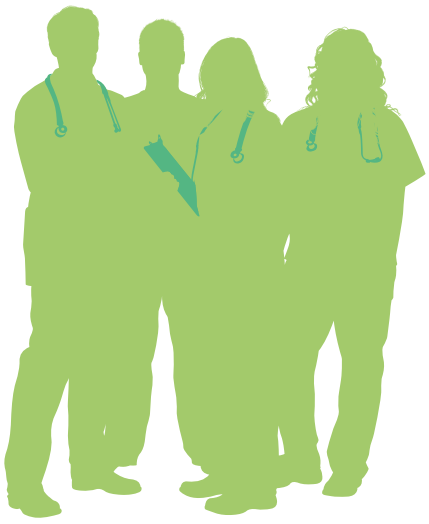
1,542
Employer Liaison Advisor meetings with employers and Responsible Officers about fitness to practise concerns

2014 — 2015

922
calls to our confidential helpline that include the raising of patient safety concerns with us

19,607
doctors we checked and approved to practise

273,761
doctors on the medical register



1,538
doctors' licences withdrawn for not engaging in revalidation

IMPROVING WHAT WE DO

Our corporate strategy

In recent years, we've transformed the way we work. Instead of waiting for problems to be brought to us, we've begun to engage much more with those working on the frontline and to listen to those who are delivering care, to help us understand where there may be risks to patient safety.

The end of 2015 marked the halfway point in our *Corporate strategy 2014–17*.* It defines our ambition to maintain the quality of our existing functions, and to continually improve so that we can be as responsive, fair and proactive as possible.

Healthcare is a complex and changing environment. The public today has different expectations and rightly demands that doctors are open and transparent in their dealings with patients and about their own performance. At the same time, pressures on doctors and other staff are increasing with many being asked to do more with fewer resources.

All of this means that organisations like ours must continue to change too.

We will continue to reflect on the progress we have made and to start developing our new corporate strategy in 2016.

* See www.gmc-uk.org/publications/corporate_publications.asp.

Our five strategic aims for 2014–17

To deliver against the ambition of our strategy we are now focusing our work on the following strategic aims.

01

HELPING TO RAISE STANDARDS IN MEDICAL EDUCATION AND PRACTICE

We will develop our standards, our guidance and the way we support particular groups of doctors to help them deal with professional challenges. We will make sure medical education equips doctors to meet these standards.

02

IMPROVING HOW WE HANDLE CONCERNS ABOUT PATIENT SAFETY

We have seen a significant rise in the number of complaints about doctors in recent years. To respond to this, we will continue to press for much-needed reform of the legislative system. We will make sure that concerns about doctors are first addressed locally wherever possible. We will continue to develop new ways to reduce the stress for those involved in our fitness to practise procedures.

03

USING INFORMATION IN SMARTER WAYS

We will use and share information in smarter ways to support high standards of medical practice and to help reduce risks to patients. Putting the information we hold to best use will help to create a more open system and to safeguard the interests of patients.

04

WORKING MORE CLOSELY WITH PATIENTS, DOCTORS AND MEDICAL STUDENTS ON THE FRONTLINE OF CARE

We will have more contact with doctors, medical students and patients so that we have a better understanding of their lives and work. More of them will be aware of our guidance and use it to help them maintain standards of patient care.

05

WORKING BETTER TOGETHER

We recognise there are often barriers to better collaboration within and between organisations – we will work across teams within the GMC to make the best use of all available knowledge and skills to help us deliver effective regulation.

Helping to raise standards in medical education and practice

It is not enough for us just to make sure that doctors perform at the minimum standard to avoid an investigation into whether they are safe to treat patients.

We want to help continually improve the standards of medical practice and maintain the confidence of the public in the profession.

We know we can't do this work alone – it's vital that we work with others, such as medical schools and local groups of doctors and patients, as well as medical royal colleges and professional associations.

Creating a single assessment for doctors who want to practise in the UK

In June 2015, our Council approved a plan to work with partners to develop a unified assessment for doctors seeking to practise in the UK. This assessment, the Medical Licensing Assessment, would involve a single assessment of those applying for a licence to practise medicine in the UK – meaning there would be a common standard for safe practice. It would replace the current Professional and Linguistic Assessments Board (PLAB) test, which international medical graduates take.

We hope that having a more straightforward and transparent route to medical practice in the UK will help make sure that all doctors practising here have been examined and evaluated to the same high level. Over time, we are confident that the Medical Licensing Assessment will help to drive up standards and that it could become an international benchmark test for entry to medicine.

Building on research, investigation and early engagement with experts and other interested parties across the UK, our Council considered an initial outline business case for the Medical Licensing Assessment. Since June 2015, we've worked closely with the Medical Schools Council and others to develop proposals for consultation, which will start in 2016. This has included engagement with the four UK governments and meetings with all UK medical schools.

We launched new standards for medical education and training

In July 2015, we launched a new single set of standards promoting excellence in medical education and training and patient safety. For the first time, we have one set of standards setting out the quality we expect of undergraduate, foundation and postgraduate training across the UK.

Promoting excellence: standards for medical education and training puts patient safety, quality of care, and fairness at the heart of the training of medical students and doctors.

The standards will be vital in making sure medical students and doctors in training are given the appropriate support in the right environment – both to learn and to provide safe and effective care for their patients. The new standards clearly set out our expectations of medical schools, postgraduate deaneries, LETBs, and local education providers when they educate and train medical students and doctors.

New work to recognise doctors' capabilities in particular practice areas

We are considering a new process called 'credentialing' to recognise doctors' capabilities in particular practice areas.* This means that a doctor who has a credential in a particular field of practice will have this recorded in their entry on the medical register. This will show they have the appropriate standards of knowledge and skills in that area.

The main purpose of this new process will be to protect patients. Being able to see a doctor's credentials on the medical register would reassure a patient that they are in the hands of a competent and capable professional.

Our consultation on the broad principles and processes for our credentialing model ran from 1 July to 7 October 2015.† We will report the results to our Council in 2016.

Development of generic professional capabilities

Working jointly with the Academy of Medical Royal Colleges (AoMRC), we have developed and consulted on a new curriculum framework for generic professional capabilities. Generic professional capabilities are broader human skills, such as communication and team working, needed by doctors to help provide safe and effective patient care.

The new framework aims to identify, simplify and clarify these important core professional capabilities that doctors should possess at the point of specialist registration. Generic professional capabilities will act as an indicative curriculum framework – they will, for the first time, explicitly state the educational outcomes needed for all postgraduate specialty medical curricula.

They will enable those that develop curricula and assessment systems, including the medical royal colleges and faculties, to integrate and contextualise the generic professional capabilities framework across the 102 specialty and sub-specialty curricula. They will also make sure common capabilities are addressed consistently across the medical workforce.

The outcome of the consultation, which ran between July and September 2015, signalled very strong support for the framework with 105 (97%) of the 108 respondents agreeing that generic professional capabilities are fundamentally important to UK medical practice. There was also good support for the framework structure generally and we incorporated some changes in response to the comments. In 2016, we will consult on revised education and training standards, which will set out explicitly the requirements for the development of curricula and assessment systems.

* 'Credentialing' is defined as 'a process which provides formal accreditation of competences (which include knowledge, skills and performance) in a defined area of practice, at a level that provides confidence that the individual is fit to practise in that area.' See www.gmc-uk.org/education/continuing_professional_development/27258.asp.

† See www.gmc-uk.org/education/27299.asp.

Setting expectations for medical professionals when things go wrong

In June 2015, we published new guidance, *Openness and honesty when things go wrong: the professional duty of candour*.* This guidance, developed in collaboration with the Nursing and Midwifery Council, sets out what is expected of every nurse, midwife and doctor practising in the UK when something goes wrong. It reminds doctors of the importance of being open and honest with patients and colleagues, and to report errors and near misses. It also aims to help patients understand what to expect from healthcare professionals.

The guidance follows Sir Robert Francis QC's call for a more open and transparent culture within healthcare, following the failures in patient care at Mid Staffordshire NHS Foundation Trust.†

In England, the professional duty of candour will sit alongside a legal obligation on organisations. A similar measure is being introduced in Scotland. It is also being considered in Wales and Northern Ireland, the latter following the publication of Sir Liam Donaldson's *The Right Time, The Right Place* report advising on improvements to governance arrangements in health and social care in Northern Ireland. We continue to work with partners across the UK on this important ethical duty.

Introducing guidance to protect patients when they have a cosmetic procedure

Between 8 June and 1 September 2015, we consulted with the public and doctors on plans for new guidance, which would contain proposals to make cosmetic procedures safer.

As a result of this consultation, we expect to publish guidance for doctors who offer cosmetic interventions in 2016.

The new guidance follows Sir Bruce Keogh's review of cosmetic interventions in England and the UK Government's response. If fulfils our commitment to develop guidance on the standards of ethical practice, working with the Royal College of Surgeons of England and others. It sets the standards we expect of all UK doctors who carry out cosmetic procedures, both surgical and non-surgical. It also aims to help patients understand what to expect from their doctor.

In Scotland, Healthcare Improvement Scotland now has the responsibility to inspect independent clinics, and we have worked with them to train their inspectors on the professional standards we expect of doctors.

* See www.gmc-uk.org/guidance/ethical_guidance/27233.asp.

† See www.midstaffspublicinquiry.com.

Supporting doctors when they care for people with dementia and mental health issues

In 2015, we updated our resources* for doctors treating older patients to include a focus on dementia and mental health issues.

The resources contain a mixture of guidance, case studies, scenarios, articles and tips to prepare doctors for caring for the growing number of older patients. We hope these materials will be useful and stimulate discussion on what are among the greatest challenges facing the profession. These resources are applicable to all doctors and we have taken care to recognise the differing legislative environments and practices across the four countries of the UK.

Reviewing our guidance on confidentiality

In 2015, we began to review our *Confidentiality* guidance and its seven supporting explanatory statements. We want to make sure they remain clear, helpful and relevant to doctors' needs, and are compatible with the law throughout the four countries of the UK.

As a first step, we ran surveys for doctors, patients and organisations that represent them, and other interested parties, such as other healthcare professionals, researchers and regulators, to ask for feedback on the content and format of our current guidance. The responses helped shape a revised draft of the guidance, which we published for public consultation in November 2015.

This is the first time our Regional Liaison Service and our new liaison advisors who are based in our offices in Northern Ireland, Scotland and Wales, have played a significant role in the consultation stage of a standards guidance development project. This approach has resulted in local tailoring of our face-to-face engagement. We will continue to work on the review of *Confidentiality* in 2016.

Updating guidance on medical students' fitness to practise

We are working with the Medical Schools Council (MSC) to revise our existing guidance, *Medical students: professional values and fitness to practise*,† for medical students and medical schools on professional values and fitness to practise.

* See www.gmc-uk.org/guidance/26081.asp.

† See www.gmc-uk.org/education/undergraduate/ professional_behaviour.asp.

Following feedback from colleagues in medical education, we've decided to split the guidance into two.

- **Achieving good medical practice:** guidance for medical students. This is a new, separate guidance document addressed to medical students to help them develop into competent and compassionate doctors that meet the expectations of patients and the public. It explains the professional values and behaviours expected of them, both on and off campus. We have mapped the guidance to the four domains of *Good medical practice*[*] – our core guidance that sets out the professional behaviours we expect of all registered doctors. We have designed the new guidance to be relevant to students, by reflecting their experiences of medical education and practice, and providing practical examples of professionalism.

- **Professional behaviour and fitness to practise:** guidance for medical schools and their students. This document will give guidance to medical schools and medical educators on processes for dealing with medical student professionalism and fitness to practise issues. This aims to offer high-level guidance on procedures, increasing standardisation and consistency while still being flexible enough to fit in with the varied governance structures of different institutions. We have also aligned it, as much as possible, with the wording and processes we use for our provisional registration and fitness to practise procedures.

We ran a public consultation on our draft versions of these two guidance documents from 19 August to 13 November 2015. This followed extensive engagement activities with those we work with, including a survey of all UK medical schools about their existing student fitness to practise processes. The findings from the medical schools' survey were presented at the annual joint GMC and MSC student fitness to practise training conference in May 2015.

We will publish final versions of the guidance in May 2016, so medical schools and students can start using it from autumn 2016.

Our mobile app for doctors' professional development

In August 2015, we successfully launched GMC My CPD – a new mobile app to help doctors manage their continuing professional development (CPD).

The app was developed and tested with input from practising professionals and is designed to support doctors to log their learning and professional development wherever they are. The app's benefits include helping doctors to keep their learning records up to date and to reflect on their learning.

The app lets users photograph learning certificates and voice record their ideas and thoughts, which can also be shared with colleagues. It also provides advice and case studies, including on planning, reflecting on points of learning, and how to prepare for appraisal. At the end of 2015, more than 20,000 users had downloaded GMC My CPD.

* See www.gmc-uk.org/gmp.

57,602 doctors revalidated in 2015.

We continued to revalidate doctors

Every five years, we ask for formal confirmation that each doctor is following our standards and that their knowledge and skills are up to date. As part of this, we work with employers and senior appointed individuals (known as Responsible Officers)* across the NHS and other medical bodies in the UK to make sure every doctor has an annual check or appraisal based on *Good medical practice* – our core ethical guidance for doctors. This system of checks is called revalidation and it helps make sure every doctor practising in the UK is competent. We're the first country in the world to introduce such an approach.

We introduced revalidation at the end of 2012 and up until the end of 2015 we had processed recommendations for 137,186 doctors. This is in line with our plans, and we are on track to revalidate the majority of licensed doctors by April 2016.

We believe that every doctor treating patients in the UK should reflect on their whole practice, both clinical and ethical, and consider how they can continue to improve their work. A doctor's Responsible Officer can tell us at any time if a doctor isn't engaging in the local systems that underpin revalidation, such as appraisal.

When we know that a doctor isn't engaging, we try to work with them. But, ultimately, if they continue to refuse to engage, we will withdraw their licence so they can no longer treat patients in the UK. In 2015, we withdrew licences from 1,538 doctors.

Helping doctors to reflect more on their practice

Appraisal rates have risen since we introduced revalidation.

A comparison of NHS England audit data† between 2011 and 2015 shows the significant increase in the number of doctors undergoing appraisal in England since the introduction of revalidation. This includes increases:

- from 79% to 93.2% of GPs undergoing appraisal
- from 64.1% to 87.3% of hospital consultants undergoing appraisal
- from 35.6% to 83.9% of staff grade, associate and specialty doctors undergoing appraisal.

In Wales, appraisal rates have significantly increased since the introduction of revalidation – from 53% in 2012–13 to 77% in 2013–14. This includes increases:

- from 88% to 92% of GPs undergoing appraisal
- from 38% to 75% of consultants undergoing appraisal
- from 22% to 68% of staff grade, associate and specialty doctors undergoing appraisal.

* See www.gmc-uk.org/doctors/revalidation/12385.asp.

† See www.england.nhs.uk/revalidation/wp-content/uploads/sites/10/2015/10/sro-report-med-revalidation-1415.pdf.

137,186

doctors revalidated since revalidation began at the end of 2012.

Healthcare Improvement Scotland's most recent national report* (2014–15) highlighted the progress Scotland is making in annual appraisal rates, which rose from 90% to 93% on the previous year. It also notes that organisations in Scotland have started to move beyond compliance with appraisal and into introducing ways of aligning revalidation with quality improvement.

Northern Ireland has also seen a significant increase in appraisal rates, with some trusts and boards now achieving 100%. The Regulation and Quality Improvement Authority is carrying out a *Review of Governance Arrangements within HSC Organisations that Support Professional Regulation*, which it will publish in 2016. This will include a review of systems that support revalidation for doctors. In addition to medical regulation, it will consider wider systems to support the regulation of professionals in social care, nursing and midwifery, pharmacy and dentistry.

Revalidation evaluation

We have appointed an independent and UK-wide collaboration of researchers, UMbRELLA† (UK Medical Revalidation Evaluation coLLAboration), led by Plymouth University Peninsula Schools of Medicine and Dentistry to carry out a long-term evaluation of revalidation.‡ It will explore the impact of revalidation and look at ways to help shape it further in the future, taking into account the different healthcare environments across England, Northern Ireland, Scotland and Wales.

The evaluation started in 2014 and we expect a series of interim reports with a final report in 2018.

In 2015, we helped the researchers carry out two UK-wide surveys: a survey of all licensed doctors (excluding those in training) and a survey of Responsible Officers – to understand their perceptions of revalidation and appraisal.

We have commissioned a review of *Taking Revalidation Forward*, which will be led by Sir Keith Pearson§ the Chair of our Revalidation Advisory Board (see page 69). Sir Keith will report his findings and recommendations to our Council by the end of 2016 and his report will be published.

Revalidation assessment

Throughout 2015, we finalised our plans to introduce the revalidation assessment in 2016.

Most licensed doctors have a connection to an organisation – a designated body – that will support them with their appraisal and revalidation. It is the Responsible Officer of this designated body who makes a revalidation recommendation about a doctor's fitness to practise.

A small number of doctors will have a connection to an approved Suitable Person. A Suitable Person is a licensed doctor who we have approved as suitable to make a revalidation recommendation.

* See www.healthcareimprovementscotland.org/our_work/governance_and_assurance/medical_revalidation/overviewlocal_reports_2014-15.aspx.

† See www.umbrella-revalidation.org.uk.

‡ See www.gmc-uk.org/doctors/revalidation/9610.asp.

§ See www.hee.nhs.uk/about-us/our-leaders-structure/hee-board.

97% of revalidation recommendations processed within five days.

Doctors who don't have a connection to a designated body or an approved Suitable Person need to sit a revalidation assessment* to show their medical knowledge is up to date. In the absence of a Responsible Officer or Suitable Person who can give us a recommendation, the assessment forms part of the evidence we need to revalidate doctors without a connection. We use information from the doctors' annual returns† (a form doctors must complete each year, which includes employment or practice history and details of annual appraisal) to make a decision about their revalidation.

From January 2016, we will begin inviting doctors to book their place to sit a revalidation assessment. The first revalidation assessments will take place in May 2016. We expect to start making revalidation decisions about these doctors around June 2016.

Helping doctors to get feedback from patients

Reflecting on patient feedback is an important part of a doctor's appraisal, but we know that collecting feedback can sometimes be challenging. In partnership with doctors and patients, we developed a set of case studies and a leaflet for patients to better support doctors with collecting feedback.

We have some evidence that patients would be more likely to give their doctor feedback if they understood how to do so and how it would be used. As a result, in 2015 we created a leaflet for patients that explains the process and how their feedback can have a positive impact on care.‡

We made changes to registration to protect patients

Introducing time limited provisional registration

We introduced a limit on the time a doctor can hold provisional registration.§ As of 1 April 2015, we allow doctors to hold provisional registration for a period of three years and 30 days.

We grant provisional registration solely so that a doctor can participate in the first year of the foundation programme (F1). Previously, doctors could hold provisional registration indefinitely.

This new limit will reduce any confusion about the kind of post a doctor with this level of registration is allowed to hold. It will also allow us to improve our protection of patients by making sure doctors are only in roles where they will be appropriately supported and supervised.

* See www.gmc-uk.org/doctors/revalidation/28041.asp for more information on the revalidation assessment.

† See www.gmc-uk.org/doctors/revalidation/23575.asp for more information on annual returns.

‡ See www.gmc-uk.org/publications/28695.asp?dm_i=2SYE,5W3E,1D0JED,HQTV,1.

§ Newly qualified doctors are entitled to hold provisional registration with a licence to practise. The purpose of provisional registration is to enable doctors to participate in and complete an acceptable programme for provisionally registered doctors. Most doctors complete this in 12 months and are then able to gain full registration with a licence to practise. See www.gmc-uk.org/doctors/registration_applications/11720.asp.

New conditions for insurance and indemnity

In August 2015, we introduced new regulations, which make it a statutory requirement for doctors to have appropriate insurance or indemnity when they work in the UK.* If we have concerns that they don't have this in place, the new regulations let us ask a doctor for information about their insurance or indemnity arrangements. We can withdraw a doctor's licence to practise, or refuse to grant them one, if they don't have appropriate arrangements or if they don't provide the information we ask for.

The regulations came into effect on 1 August 2015 and, following a series of communications in advance of the changes, we wrote to all doctors in September 2015 to explain the changes.

Professional and Linguistic Assessments Board (PLAB) test

We are making improvements to strengthen the PLAB test. The test is the main route by which international medical graduates demonstrate that they have the necessary skills and knowledge to practise medicine in the UK.

In 2014, an independent review,† which we commissioned, confirmed the PLAB test was fit for purpose. But it also made a series of recommendations to improve it, which we accepted.

In early 2015, we began a programme of work to implement these recommendations. We will introduce the new strengthened assessment in September 2016 and there will be ongoing work through the Medical Licensing Assessment (see page 16 for more information).

Improving how we handle concerns about patient safety

We often receive enquiries from patients and members of the public seeking to complain about all parts of the healthcare system and other medical professions. In those instances, we always explain that we can only investigate complaints about doctors and then direct them to the organisation(s) best placed to help them with their complaint.

In recent years, the number of complaints we have received about doctors has risen significantly. In some cases, it is clear from the outset that there is no need for us to investigate because the complaint is about matters that cannot raise an issue of impaired fitness to practise. We will normally close these cases without taking any further action. Examples of cases closed without any investigation are:

a minor motoring offences not involving drugs or alcohol

b a delay of less than six months in providing a medical report

c a minor non-clinical matter

d a complaint about the cost of private medical treatment.

We also conclude many other cases after discussing the concerns with the doctor's employer or contractor. By checking with the employer or contractor, we can satisfy ourselves that a complaint is not part of a wider pattern of concerns. Where the

* See www.gmc-uk.org/doctors/information_for_doctors/insurance_and_indemnity.asp.

† See www.gmc-uk.org/05___Report_of_the_Review_of_the_Professional_and_Linguistic_Assessments_Board_test.pdf_57874921.pdf.

employer or contractor reassures us that there are no grounds to investigate further, and the matters complained of are not, in themselves, serious enough to require us to take any action, the case will be closed.

Examples of cases closed after we have checked with the doctor's employer that they have no concerns are:

a complaints about the quality of treatment received where there is no indication of any risk to the patient or that the doctor acted significantly below appropriate standards

b complaints about doctors' poor attitudes to patients, or failing to take their preferences into account.

For the remainder of cases, we carry out an investigation into the doctor's fitness to practise before we decide what action to take. This may include taking witness statements, obtaining an expert report, or undertaking an assessment of the doctor's health or performance. We must then decide whether we should conclude the case with no further action, issue a warning, offer the doctor undertakings or refer the doctor for a hearing by a medical practitioners tribunal.

Between 2010 and 2015, the volume of complaints about a doctor's fitness to practise increased by 32% from 2010, and by 7% from 2011. Not all complaints about doctors are serious enough to meet our thresholds for full investigation. But within this same period, the volume of complaints that met our threshold for investigation also rose, by 15% from 2010, and by 2% from 2011. For more information about incident location for fitness to practise concerns please see page 47.

Other health and social care regulators have also seen increases in the number of complaints about their registrants. Part of the challenge in dealing with concerns efficiently is that the current legislative frameworks for all health and social care regulators are expensive, complex and need continuous government intervention to keep them up to date. This means that, if we want to make a change to our processes, we must follow a lengthy legalistic process that prevents us from being as responsive to changes as we need to be. We'll continue to press for urgently needed law reform.

Legal changes to strengthen our investigations and tribunal service

In the meantime, we've continued to seek improvements within the existing legislative framework. On 31 December 2015, changes to the *Medical Act 1983* established the Medical Practitioners Tribunal Service (MPTS) on a statutory basis. The changes have also given us the power to appeal decisions MPTS tribunals make if we think they don't sufficiently protect the public – a power that didn't previously exist. We introduced operational changes to improve the efficiency and effectiveness of MPTS hearings during 2015, including our right to appeal, connected with this legislative change.

The landmark law changes will also reform the way our investigations and MPTS hearings are handled. These include:

- robust powers to manage hearings, including giving the MPTS the power to award costs if either the GMC or the doctor fails to comply with directions and behaves unreasonably in the conduct of proceedings

- powers to have a legally qualified chair instead of a chair and legal assessor in some hearings.

In 2015 we also introduced a change to our triage process which allows us, in certain cases, to obtain key information before deciding if a full investigation is necessary. We call this a preliminary enquiry – in 2015, this helped us avoid opening 218 full investigations.

We also introduced meetings with doctors and their representatives at the end of an investigation in some cases. This let us communicate our concerns directly and clarify any information the doctor needed to provide before we made a decision about the outcome. In 2015, this helped us avoid a referral to a hearing in 31 cases.

Improving support for those who make a complaint

Our pilot of patient meetings has received a significant amount of positive feedback from those who have participated. Feedback from the independent evaluation makes it clear that the meetings build rapport with complainants and reduce their sense of isolation within our fitness to practise process, and gives us opportunities to clarify their concerns. Complainants also said the meetings help them to understand our role, purpose, investigation process and what to expect. This helps to manage their expectations.

Given this feedback, in 2015 we rolled out accessible meetings with patients across the UK. We now offer face-to-face meetings in the Manchester and London offices, as well as in our Cardiff, Edinburgh and Belfast offices.

Supporting doctors involved in our fitness to practise processes

In 2015, we started work to address the recommendations of an independent review of cases where doctors have taken their lives while in our fitness to practise procedures, which was published at the end of 2014.*

We reviewed the tone of the letters we write to doctors we are investigating – working with the British Medical Association (BMA), medical defence organisations and other groups that support doctors. The new letters have received positive feedback. We also appointed a leading mental health expert, Professor Louis Appleby, to oversee a fundamental review to identify improvements and reduce the impact of our approach to investigating concerns about doctors who are vulnerable. This review will continue in 2016.

Alongside this work, we designed training for our staff to raise awareness of mental health concerns. In addition, we have published new standards for training providers, requiring medical education and training organisations to provide a supportive environment and culture to learners, and a set of generic professional capabilities to equip doctors for medical practice, as discussed on page 17.

* See mc-uk.org/Internal_review_into_suicide_in_FTP_processes.pdf_59088696.pdf.

A new service to give doctors independent support and advice

We understand some doctors find it difficult and stressful to have their fitness to practise investigated. After a successful pilot, we launched the Doctor Support Service in 2015. It is run by the BMA's Doctors for Doctors to provide dedicated confidential and emotional support for doctors. This service is free and open to any doctor involved in a fitness to practise case.

The service offers a range of support. Doctors can receive emotional support from a fellow doctor who is independent of the GMC. If their case ends up at a hearing, a member of the service can go with them for up to two days and, once a case has closed, doctors can talk to their supporter about how they feel about the outcome.

Meetings with doctors to improve information sharing

In 2015, we implemented meetings with doctors following a successful two-year pilot. The aim of the meetings is to improve the way we and the doctor share information. We offer to meet with the doctor at the end of an investigation to explain which factors of the case are of most concern. The doctor then has the opportunity to share information with us that we may not already be aware of and that may help us to understand the case better. This helps us determine the most appropriate course of action and identify whether a hearing is needed.

We understand that hearings can be stressful for witnesses and doctors. By sharing information at an earlier stage through the meeting process, we can avoid unnecessary hearings, speed up the resolution of cases and reduce the stress for all involved. The meetings are not open to all doctors, but those who are likely to be referred to a tribunal hearing. The meetings are optional and don't affect our view of the case.

Doctors who have attended a meeting report their understanding of our key concerns had improved and this helped them to provide a fuller, written response to the allegations. Doctors also say they value the opportunity for constructive and open discussion about their case.

The meetings take place in our London and Manchester offices and are run by a senior member of staff, called a case examiner, who will make the decision on the case. A member of our legal team will also be there. The doctor may bring a representative from their defence organisation, a relative, friend or a doctor from the Doctor Support Service for support.

Improving our guidance for tribunals on sanctions

In 2015, we introduced changes to the joint MPTS and GMC Sanctions guidance* – the guidance decision makers use to decide how serious a breach of our standards is, and what action should be taken. The new documents give more-detailed guidance on matters that should lead to outcomes that are more serious.

We made these updates following a major consultation in 2014, which received the largest number of responses to any GMC consultation. It asked for views on various proposed changes to make sure decisions are fair and consistent. The revised guidance sets out more clearly:

- what action to consider in more serious cases – for example, where there has been misconduct involving the misuse of drugs or alcohol, discrimination against patients or colleagues, or violence towards others
- to what extent personal consequences for doctors should be considered when taking action to protect patients
- what type of behaviour demonstrates a doctor has insight – for example, if they apologised to the patient.

Changes to the information we publish and disclose about a doctor's fitness to practise

We consulted on our proposal to introduce time limits for how long sanctions on a doctor's registration are published online or made available to the public.

Under current rules, when a doctor receives a sanction the outcome is published on our website and disclosed to anyone who asks. It remains online indefinitely, even after any restrictions have been lifted.

This includes details of doctors who have been suspended, had restrictions on their practice, agreed undertakings such as retraining or been erased from the medical register altogether.

* See www.gmc-uk.org/concerns/hearings_and_decisions/sanctions_referrals_guidance.asp.

The consultation ran from July to September 2015* and covered six main areas:

- the introduction of time limits on the disclosure of fitness to practise information on our online register
- the transfer of historical data onto the online register – fitness to practise information from before our online register was introduced in October 2005
- publishing information about the outcomes of fitness to practise appeals
- clarifying our policy on publishing information about appeals of interim orders
- providing a greater explanation of our decision where a case concludes with a doctor agreeing undertakings without a fitness to practise tribunal hearing
- whether our proposals would affect people with protected characteristics covered by equality legislation.

The consultation results are helping to develop our new policies, which are set to be introduced in August 2016.

Our decisions are fair and consistent to doctors under investigation

In March 2015, the report *Review of decision-making in the General Medical Council's Fitness to Practise procedures*† was published.

We commissioned researchers at Plymouth University to review how we made decisions in our investigations of doctors who were the subjects of a serious complaint during 2014. This review is part of an ongoing programme of work to make sure our fitness to practise procedures are fair, consistent and robust.

The key findings were:

- the decisions reached by GMC staff in all 187 reviewed fitness to practise case files were found to be appropriate and in line with the guidance and criteria set out for decision makers
- there was no evidence of bias or discriminatory practice, either in the wording of our guidance and criteria documentation for decision makers or in the sampled case files, that might explain differences in outcomes for certain groups of doctors

* See www.gmc-uk.org/Final_consultation_outcome_report_reformatted__2_.pdf_64891740.pdf

† See www.gmc-uk.org/about/research/26342.asp.

- previous research exploring why certain groups are more likely to be involved in formal disciplinary procedures have not found the underlying causes as to why. The current evidence suggests, however, that employers may be more likely to refer groups with certain demographic characteristics into formal procedures or these groups may be more likely to work in roles that are prone to a greater number of complaints

- in some of our cases, the decisions made by case examiners were not as fully reasoned as they could be, although the decisions themselves appeared appropriate for the circumstances of the case. Furthermore, the distinction between concluding a case and concluding a case with advice was unclear in a few cases.

Since the report, we have given our case examiners enhanced training on writing decisions and made changes to our approach to issuing advice.

How we deal with complaints about doctors who are whistleblowers

We asked Sir Anthony Hooper to conduct an independent review of how we deal with doctors referred to us, and who may have raised concerns in the public interest.*

Sir Anthony Hooper's review was published in March 2015. It included eight recommendations to better support whistleblowers, to encourage an open and transparent culture and to make sure our procedures are fair to those who raise concerns in the public interest.

In August 2015, we published our response to the review in our action plan,† which outlines how we will consider each recommendation in his report. Throughout the remainder of the year we:

- considered how we can encourage employers and Responsible Officers to confirm whether the doctor being referred is a whistleblower

- reviewed the processes and guidance we follow when we receive a complaint about a doctor, so we can check if he or she has raised concerns and take this into account

- started work to develop training for our teams about whistleblowing

* See www.gmc-uk.org/Hooper_review_final_60267393.pdf.

† See www.gmc-uk.org/Hooper_Review___Action_Plan.pdf_62048557.pdf.

- planned an event to facilitate discussion with other regulators and organisations about a confidential online tool so healthcare professionals can record concerns raised.

We will continue with this work throughout 2016.

In addition, in Northern Ireland we participated in the Regulation and Quality Improvement Authority's *Review of the Operation of HSC Whistleblowing Arrangements*, which will be published in 2016.

Using information in smarter ways

We are just one part of a complex health system and have a role to play in working with others to identify and act on risks to patients. Understanding and sharing data – both within our own organisation and with others – are important in helping us to identify risks and to plan the right action to take in response.

Sharing information with other regulators

We work with systems regulators* and improvement bodies in all four countries of the UK to ensure the best use of shared intelligence† for patient safety and local improvement. In 2015, we built on our existing arrangements with the Care Quality Commission in England to sign Memoranda of Understanding‡ with Healthcare Improvement Scotland and the Regulatory and Quality Improvement Authority in Northern Ireland and an Information Sharing Agreement with Healthcare Inspectorate Wales.

Sharing our data to improve healthcare and transparency

On 17 November 2015, we published our fifth annual report of *The state of medical education and practice in the UK*.§ The 2015 report uses our data and the data of other organisations to assess the trends, opportunities and challenges facing medical education and practice across the UK.

The 2015 report focused on a number of areas, including the changing workforce and patterns in global migration of doctors joining and leaving the profession. We also conducted research on the most serious fitness to practise cases to give us a better understanding of these types of cases.

* Systems regulators are organisations, such as the Care Quality Commission in England, the Healthcare Inspectorate Wales, the Regulation and Quality Improvement Authority in Northern Ireland and Healthcare Improvement Scotland which are responsible for overseeing the quality of care provided by organisations across the UK.

† We define 'intelligence' as information that we have gathered and assessed to help support and improve our decision making and performance.

‡ A Memorandum of Understanding (MOU) is a statement agreed by two or more organisations which commits them to working closely together to support each other's goals. See www.gmc-uk.org/about/partners_index.asp.

§ See www.gmc-uk.org/somep2015.

Using our information about patient safety concerns

We hold data and information about individual doctors, fitness to practise complaints and education and training environments. Our *Data Strategy* project allows us to manage this data and use it more effectively, across all our functions, to further patient safety. The fundamental elements of the work include:

- an internal database and interactive reporting system
- the pilot of a UK-wide Medical Education Database* delivered in partnership with a range of medical education bodies including NHS Education for Scotland, The Wales Deanery and Northern Ireland Medical and Dental Training Agency
- a new common data set for the environments that doctors work in across the UK.

We plan to make much of this data available to groups we regularly work with in 2016.

Alongside this work, we are also developing an intelligence model to help us use our data to build a better understanding of the risks in medical practice and to use our resources more effectively and proportionately. This work will let us filter and combine our data with external information, so we can get an insight into specific areas of risk or harm.

Using our data and intelligence to help make sure we are a fair organisation

We also use our data and intelligence to make sure that, as well as meeting our legal obligations, we're proactive in understanding the factors that affect different groups.

In 2015 our report, *The state of medical education and practice in the UK*, identified that complaints referred to us by the police and by employers of doctors were more likely to be for black and minority ethnic doctors than were complaints from other sources. We are currently considering the implications of this, and have commissioned research into what processes lead to someone referring a doctor to us.

The report also highlighted that some doctors whose complaints lead to a warning, conditions or undertakings can encounter barriers when they return to practice. The research found that doctors were most successful where they have the support of a sympathetic employer, and where the doctor feels the complaint was justly raised and their voice was heard during the fitness to practise process.

* See www.ukmed.ac.uk.

In March 2015, we published two interactive reports on the progress of doctors in training,* based on recruitment data showing doctors applying for specialty and GP training programmes, and examination data showing pass rates. The reports illustrate that some groups of doctors in training are less likely to progress than others. We are working with those involved in managing and delivering medical education to investigate the underlying causes.

In March 2015, we also published our report *Building a supportive environment: a review to tackle undermining and bullying in medical education and training*.† It highlights areas where doctors believe bullying and undermining is more common. The review focused on obstetrics and gynaecology, and surgery, as these specialties were identified as among those having the most issues with bullying and undermining.

Continuing to develop our online medical register

We are committed to making the online medical register more useful and accessible for employers, doctors and the public.

In 2015, we made some small changes to the register, also known as the List of Registered Medical Practitioners (LRMP), so that it reflects our changing responsibilities. It now shows:

- which doctors are in approved training programmes
- which programme specialty they are in
- their deanery in Northern Ireland, Scotland or Wales or their local education and training board in England
- a licensed doctor's designated body (or Suitable Person, if they have one) and Responsible Officer
- which doctors are recognised as GP trainers across the UK countries.

We believe there is a public interest in developing the online register to show more information about a doctor's practice and we are committed to a more fundamental consultation in 2016 on the future of our online register to help determine future changes.

* See www.gmc-uk.org/Briefing_note___Exams_and_recruitment_outcome_reports.pdf_60060997.pdf_60086828.pdf_62012305.pdf.

† See www.gmc-uk.org/education/26979.asp.

Working more closely with patients, doctors and medical students in frontline care

Our ability to successfully engage doctors in raising standards relies on us working more closely with others, including patients, educators, employers, medical students, and other healthcare professionals and regulators. By understanding more about the daily challenges that doctors face, we can learn more about what motivates and influences their behaviour and engage them more successfully.

We sought to understand and help doctors to overcome some challenges they face in the workplace

In 2011, our first report on *The state of medical education and practice in the UK* found evidence that doctors new to UK practice needed more support. In 2015, we continued to roll out our Welcome to UK Practice programme,* which was developed in 2013 as a response to our 2011 findings that doctors new to practice would benefit from this kind of support.

In 2015, there was a 285% increase in the number of attendees at Welcome to UK Practice events across the UK compared with 2014. We ran and evaluated pilot sessions for international medical graduates before they joined the medical register – 99% of participants said they would change their practice as a result of taking part.

We believe that this support for doctors new to practising in the UK can help improve their understanding of how to apply our standards and the quality of care that patients receive in the UK.

Our Regional Liaison Service and offices in Northern Ireland, Scotland and Wales allow us to listen to and understand the needs of doctors, patients, medical students and other key groups across the UK, and have been fundamental to delivering events in 2015.

Resources designed to support doctors new to UK practice are also available on our website. Our online scenario-based tool† allows doctors to assess their knowledge and application of our core guidance, *Good medical practice*. It contains 22 different case studies that explore ethical challenges in a variety of clinical scenarios. It also provides help on finding further information and advice.

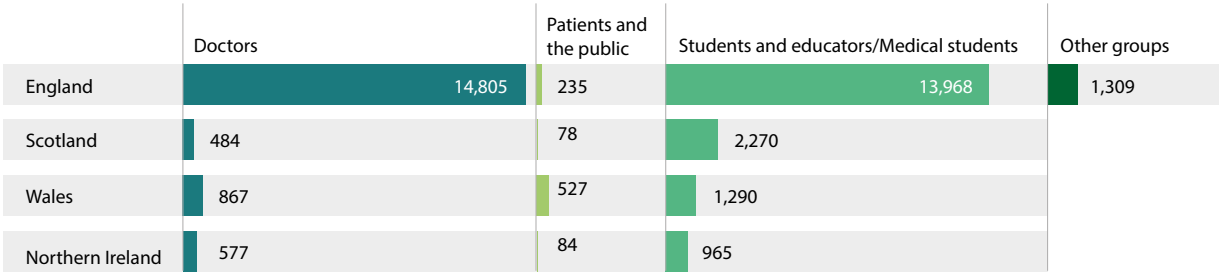
We listened to the needs of doctors, patients and medical students and others across the UK

The events run by our Regional Liaison Service and our offices in Northern Ireland, Scotland and Wales give us an opportunity to discuss the work we do and to listen to and understand the needs of those we work with and for. We use the feedback from this engagement to develop our policies and systems and to make sure they reflect the realities of practice in the context of the law, healthcare systems and structures in each country of the UK.

Our UK-wide Employer Liaison Service‡ supports a two-way exchange of information about underperforming doctors, improving patient safety and the quality of referrals. In 2015, it held more than 1,420 meetings with Responsible Officers. The meetings help Responsible Officers and their teams to understand our fitness to practise thresholds and procedures and help us help us understand issues that arise locally, regionally and from across the four UK countries.

* See www.gmc-uk.org/doctors/WelcomeUK.asp.
† See www.gmc-uk.org/guidance/case_studies.asp
‡ See www.gmc-uk.org/els.

Engagement with key interest group by country in 2015



Our Good Doctors website

In 2015, we launched *Medical professionalism matters*, which is a programme we developed in partnership with an advisory group of healthcare organisations, that we lead, to help us better understand the challenges that health professionals face in their daily practice.

The programme consists of a series of events and online discussions, which enable frontline clinicians to discuss a range of topics focusing on some challenges posed by 21st century medical professionalism.

As part of the *Medical professionalism matters* programme, we launched a new website, www.gooddoctors.org.uk, for doctors and others involved in medicine to discuss and explore the challenges they face.

The site features blog posts, polls, photos and videos from our events. We also bring together content inspired by online and in-person discussions about medical professionalism, taking these discussions to a wider audience.

Working better together

It's essential that we continue to give clear value for money, especially as demand for our services increases and pressures within the health service continue. This includes finding new ways of doing things by improving our existing approach, and by exploring better ways of working together, both internally and with external partners.

Our Chief Executive, Niall Dickson, became Chair of the International Association of Medical Regulatory Authorities (IAMRA) in October 2014 for a two-year term. In 2015, we have used our leadership of IAMRA to promote medical regulation as a means of protecting patients throughout the world.

We continue to contribute to the work of IAMRA through active participation in the committees of the association and through events such as the Third International Revalidation Symposium in Montreal, Canada.

We continue to reflect on and improve our four-country regulatory model

Since they were established, in 2003 (Scotland) and 2005 (Wales and Northern Ireland), our offices in these countries have largely grown organically. The scope of the offices has expanded to support a much wider portfolio of colleagues and teams as our role and size have expanded and diverging policy issues have emerged in each country.

In 2015, yet again, we saw further divergence in the UK political landscape, healthcare delivery models and the consequent practice of medicine across England, Northern Ireland, Scotland and Wales. Reflecting on this and considering the time since these offices opened, in 2015 we completed a review* to better understand how each office was aligned to our strategy and business plans.

The review found that those with an interest in our work benefited from having a local presence to communicate with the wider GMC. A local presence also helps us to give due consideration of the local political, social, legal and healthcare landscape. The review's findings reaffirmed our commitment to maintaining our four-country presence and the importance of our original strategic decision to pursue four-country representation.

The review helped us to think about the strategic and operational challenges we might face in the future to maintain relevance to our audiences and partners in Scotland, Wales and Northern Ireland. It has also helped us consider the dynamic context we work within – the increasingly complex UK-wide regulatory model and the divergent healthcare landscapes across the UK. Our next corporate strategy will incorporate our findings following this review.

Changes to legislation affecting our work

UK legislative reform and the future of regulation

For some time, we have campaigned for major reform to the legislation that underpins our work. In December 2015, the UK Government announced in a written ministerial statement† that it would not take forward the Law Commissions' Bill‡ as originally envisaged. Instead, it proposed to take a new, and potentially more radical, look at the future shape of professional regulation with a view to producing a new Bill. We understand the current plan is for a public consultation on proposals in autumn 2016 with a view to introducing legislation in the 2018–19 session of this Parliament.

The current timings may well change but we will engage with the four governments of the UK, with other regulators and with our key interests to help shape the proposals for reform.

* See www.gmc-uk.org/03___Devolved_Office_Review.pdf_65766618.pdf.

† See www.parliament.uk/business/publications/written-questions-answers-statements/written-statement/Commons/2015-12-17/HCWS417/.

‡ See www.lawcom.gov.uk/project/regulation-of-health-and-social-care-professionals/#regulation-of-health-care-professionals-regulation-of-social-care-professionals-in-england.

We continued to work closely with the Scottish Government

In 2015, our team in Scotland worked effectively with the Scottish Government on a number of legislative and structural changes. We participated in the development of the Scottish Government's Strategic Framework for Action on Palliative and End of Life Care, which aims to make sure everyone in Scotland who needs palliative care will have access to it.

2015 saw the passage of a new *Health (Tobacco, Nicotine etc. and Care) (Scotland) Act 2016* through the Scottish Parliament. We provided written advice on two aspects of the Act relating to our role: the introduction of the offences of ill-treatment or wilful neglect in health and care settings, and the introduction of an organisational duty of candour. We are working closely with the Scottish Government on the implementation of the new duty so that it can help foster a culture of openness and honesty, and build on doctors' professional duties.

The Scottish Parliament also passed the *Apologies (Scotland) Act 2016*, which will mean apologies will not be liable as evidence in most civil proceedings in Scotland. Following close work between the GMC, the Nursing and Midwifery Council and the Scottish Government, the Minister for Community Safety and Legal Affairs, Paul Wheelhouse, told the Scottish Parliament of his intention to add an exception into the Act for proceedings for proceedings held by health professional regulators. This would safeguard our ability to protect patient safety by allowing us to assess a doctor's insight where an apology, or lack of an apology, may have been a determining factor.

We continued to work closely with the Welsh Government

In 2015, our team in Wales continued to engage and work effectively with the Welsh Government on a number of structural and legislative changes. Throughout the year, we responded to a series of consultations, including those on the Welsh Government's Health Standards Framework, the Health Professionals Education Investment Review and the NHS Wales Workforce Review.

In addition, we provided a verbal and written response to the Our Health, Our Health Services Green Paper, which aims to introduce a wide range of reforms to the NHS in Wales, including the reconstitution and statutory separation of Healthcare Inspectorate Wales from the Welsh Government. Colleagues from other directorates also helped in providing a response to the *Mental Health Act Code of Practice* consultation.

Scotland Act 2016 and future Wales Bill

We maintained open dialogue with the Scottish Government during the consideration of the *Scotland Act 2016*, to make sure that both we and the UK and Scottish Governments fully understood any potential impact on professional healthcare regulation of proposals to devolve powers. The Act does not devolve any powers that impact directly on professional healthcare regulation. We will take a similar approach with the Welsh Government when the Wales Bill is introduced (expected in 2016). We do not expect the Bill to contain any proposals that impact directly on professional healthcare regulation.

Monitoring legislative developments in Northern Ireland

In 2015, we closely monitored a number of key pieces of legislation as they progressed through the Northern Ireland Assembly, which could affect medical practice or regulation.

The *Mental Capacity Bill,* when enacted, will introduce a single statutory framework governing all decision making in relation to the care of a person aged 16 or over, who lacks capacity. The Bill fuses mental health and mental capacity legislation, which has not been attempted in any other jurisdiction and is set to be passed in 2016. We will continue to liaise with the Department of Health, Social Services and Public Safety as it takes forward the implementation of the Bill over the next number of years.

2015 saw further discussion of abortion legislation in Northern Ireland. The *Abortion Act 1967* does not apply in Northern Ireland and, in December 2015, the High Court in Belfast ruled that abortion legislation in Northern Ireland is incompatible with the UK's obligations under the *Human Rights Act 1998*. This ruling has been appealed by the Northern Ireland Attorney General and Northern Ireland Justice Minister. The Minister has committed to publishing new guidelines for health professionals, clarifying existing legislation in Northern Ireland in 2016. We will continue to monitor this, and any further proposed legislative development or court rulings, for any potential impact on our existing guidance for doctors.

In 2015, the *Public Services Ombudsman Act (Northern Ireland) 2016* established the Office of the Northern Ireland Public Services Ombudsman, combining the previous offices of the Assembly Ombudsman for Northern Ireland and the Northern Ireland Commissioner for Complaints. The Act introduces a number of new powers including the power for the Ombudsman to investigate complaints in a range of areas, including maladministration in the exercise of administrative functions by healthcare providers and in clinical judgement in healthcare.

We successfully engaged on European regulation to improve patient safety

We have to comply with the relevant European legislation affecting medical regulation such as *Directive 2005/36/EC* on the recognition of professional qualifications (RPQ). This sets out our obligations for facilitating the freedom of movement of professionals by recognising the medical qualifications held by doctors from within the European Economic Area. Amendments to the directive were agreed by member states in 2013 and came into force in January 2016.

Our work on the implementation of the directive began in March 2014 with the establishment of the cross-directorate, monthly RPQ implementation working group. Throughout 2015, ongoing liaison with the European Commission, UK Government officials and other UK and European regulators made sure our concerns and priorities were considered and we successfully implemented the changes by the required date.

This included responding to both UK and EU public consultations and ongoing liaison with the Department of Health on the draft amendments to the *Medical Act 1983*. We worked as part of our UK and European networks of regulators to make sure doctors were not included in the first phase of the European professional card and successfully influenced the content of the European legislation implementing both this and the alert mechanism.* Through a number of face-to-face meetings with UK Government officials and the European Commission, we influenced the design of the European-wide alert mechanism. And we worked to gain a commitment that a full evaluation of the European professional card is carried out in 2016.

Our change programme

To make sure we continue to deliver a model of regulation that is fit for purpose for the future, our Council asked our Chief Operating Officer to conduct a thorough review of the organisation from autumn 2014 to autumn 2015. The review encompassed five main areas of focus:

- organisational culture
- organisational design and capability
- activity and demand
- our cost base
- our income streams.

The outcomes of this review were that we needed a focused and thorough programme of transformational change to help us deliver an ambitious corporate strategy, cope with rising demand for our services, become more responsive and flexible as a regulator and identify and act on risks to patient safety in an evidence-led and risk-based way.

We also identified a need to become even more efficient and effective to be able to demonstrate value for money for our registrants and good stewardship of our resources in delivering our statutory and charitable functions for public benefit.

As a result of these findings, we established our change programme. It focuses on five key portfolios of work that will enable us to deliver the changes we need as a business while also helping us safeguard our future financial sustainability. The portfolios are:

- organisational design and capability
- income generation
- reducing costs
- working smarter
- innovation.

* See www.ec.europa.eu/internal_market/imi-net/important_documents/alert_mechanism_en.html.

The change programme was approved by our Council in 2015 and we have already started work on the five portfolios. As part of the programme, we will be reducing the size of our London office during 2016 by moving approximately 140 posts up to our Manchester office and making approximately 20 posts redundant. We continue to communicate, consult and support staff during this period.

As part of the change programme, we are also developing a new area of work called *GMC Services*. As the medical regulator we have expertise in a number of areas, which puts us in a strong position to provide services for other organisations, such as overseas regulators, who would welcome our support. We want to build on services we already provide for free so that in future we can cover our costs or derive an income from these activities.

We continue to implement our Welsh Language Scheme

While we await the introduction of the new Welsh language standards,* we continue to implement our Welsh Language Scheme. This has included submitting an annual report to the Welsh Language Commissioner in summer 2015 and meeting with the Commissioner to discuss progress towards the standards.

We are also making sure we continue to embed the scheme and the principle of thinking through the linguistic consequences when we create new policies or processes, procure or launch new services and discuss these changes in our governance meetings. We made sure that consultation documents for three new patient interest initiatives in 2015 – on duty of candour, education standards and fitness to practise rules – were available in Welsh and continue to embed the scheme through our internal policy network and procurement strategy.

One important operational change we agreed is the introduction of on-the-phone translation for Welsh speakers in our contact centre and Cardiff office. This will be available from May 2016.

* See www.comisiynyddygymraeg.cymru/English/Language%20duties/Pages/What-are-standards.aspx.

We have established an Assessment Advisory Board

We have established an Assessment Advisory Board to provide expert advice on the development and operation of our assessments and assessments that we oversee. It will report its work to our Strategy and Policy Board.

A lay member and a range of experts in assessment and assessment design, including those with expertise in differential attainment and statistical analysis of examination performance (psychometrics), have joined the board under the leadership of Professor Val Wass. Members are appointed in a personal capacity and do not represent any organisations.

The first substantive meeting of the board was in October 2015.

We continued our work on being a fair employer

In 2015, we continued our work on being a fair employer. Our aim is to treat everyone who works with us with dignity and respect and to work towards being a diverse workforce at all levels of the organisation. This includes taking steps to make sure our policies, recruitment and selection processes continue to align with our commitment to being fair.

At the end of 2015, we employed 1,207 staff (1,113 permanent or fixed term). Our aim is to achieve a diverse workforce at all levels. In terms of ethnicity, we continue to attract a high percentage of applicants for roles from a black and minority ethnic background (28% in 2015). Areas of progress include the sustained increase in the number of female managers and a positive attitude toward diversity, for example 50% of our senior managers are women and 10% of all our managers are from a minority ethnic background.

OUR STATUTORY OBLIGATIONS

In addition to the new developments and innovation we are striving for in our *Corporate strategy 2012-17*, we also have to make sure we deliver a consistent, efficient and fair service against our statutory obligations.

We are an independent organisation. We help protect patients and improve medical education and practice across the UK by delivering our core functions.

- We set the standards that doctors need to follow, and make sure they continue to meet these standards throughout their careers.
- We decide which doctors are qualified to work here and we oversee UK medical education and training.
- We take action to prevent a doctor from putting the safety of patients, or the public's confidence in doctors, at risk.

In this section, we explain how we achieved this in 2015.

Making sure our role and standards are understood

Our standards define what makes a good doctor by setting out the professional values, knowledge, skills and behaviours required of all doctors working in the UK. The core professional standards expected of all doctors are set out in our guidance, *Good medical practice*.

This guidance covers the fundamental aspects of a doctor's role, including working in partnership with patients and treating them with respect. We also provide detailed guidance on the ethical principles that most doctors will use every day.

We want to do everything we can to make sure our guidance is widely known, understood, and applied by doctors in their day-to-day lives. Serious or persistent failure to follow our guidance will put a doctor's registration at risk. Our Regional Liaison Service and our offices in Northern Ireland, Scotland and Wales played a key part in 2015 in raising awareness and understanding of our standards.

In 2015, we directly engaged on our role across the UK with:

- 16,733 doctors
- 924 patients and members of the public
- 18,493 students and educators.

Of those we engaged with:

- 100% of key partners surveyed said working with us would improve the practice of doctors they work with
- 96% of doctors and medical students surveyed said they would reflect on their practice as a result of attending a Regional Liaison Service event
- 95% of doctors and medical students surveyed said they would reflect on their practice as a result of attending an engagement event run by one of our offices in Northern Ireland, Scotland or Wales
- 76% of doctors and medical students surveyed they would change their practice as a result of attending a Regional Liaison Service engagement event
- 66% of doctors and medical students surveyed they would change their practice as a result of attending an engagement event run by one of our offices in Northern Ireland, Scotland or Wales.

In Northern Ireland, we have been working closely with the postgraduate deanery, the Northern Ireland Medical and Dental Training Agency (NIMDTA), to deliver a professionalism module as part of their iQuest (Improving Quality and Understanding to Enhance Specialty Training) programme. This module is aimed at those in the initial years of specialty training and will be made compulsory from August 2016. The module will focus on:

- good medical practice
- doctors' use of social media
- consent
- confidentiality
- protecting children and young people
- treatment and care towards the end of life
- raising and acting on concerns about patient safety
- professionalism.

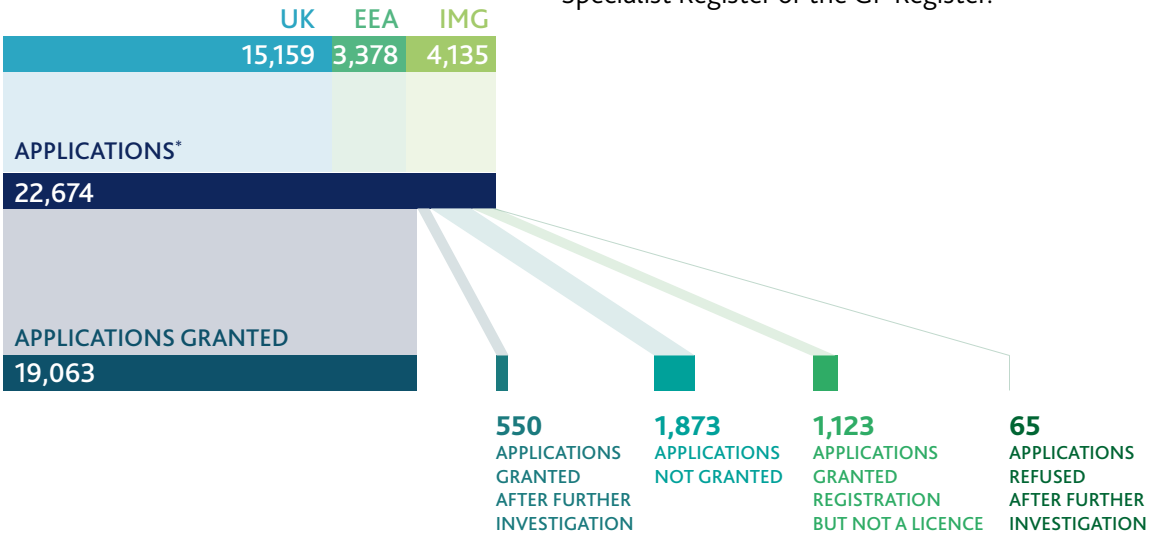
You can find out more about our four-country engagement in the *Working more closely with patients, doctors and medical students in frontline care* section on page 34.

Registering doctors who are safe and fit to practise

We check each doctor's identity and qualifications before we allow them to practise in the UK. We maintain a list of all the doctors that meet our requirements – this is called the medical register.

In 2015, we processed 22,674 applications for registration.* Of these, 15,159 applications were from doctors who qualified in the UK, 3,378 in the European Economic Area (EEA) or Switzerland, and 4,135 from the rest of the world, also known as international medical graduates (IMGs). We approved 19,607 doctors for registration with a licence to practise.

2015 outcomes of applications for registration



We gained powers in 2014 to check the English language skills of European doctors who apply for a licence to practise medicine. This helps us to make sure that only doctors who are able to communicate effectively in English are able to treat patients in the UK. In 2015, 736 doctors from Europe did not satisfy our English language requirements when they applied for entry to the medical register or afterwards in 2015. These doctors were not licensed and could not practise in the UK.

We also maintain a Specialist Register and a GP Register. These list the doctors who have completed further specialty training and who meet the criteria to work as a GP in the NHS or to be appointed in a substantive consultant role in the NHS, or health and social care sector in Northern Ireland. In 2015, we granted over 8,000 applications (including new applications and restorations) for entry onto the Specialist Register or the GP Register.†

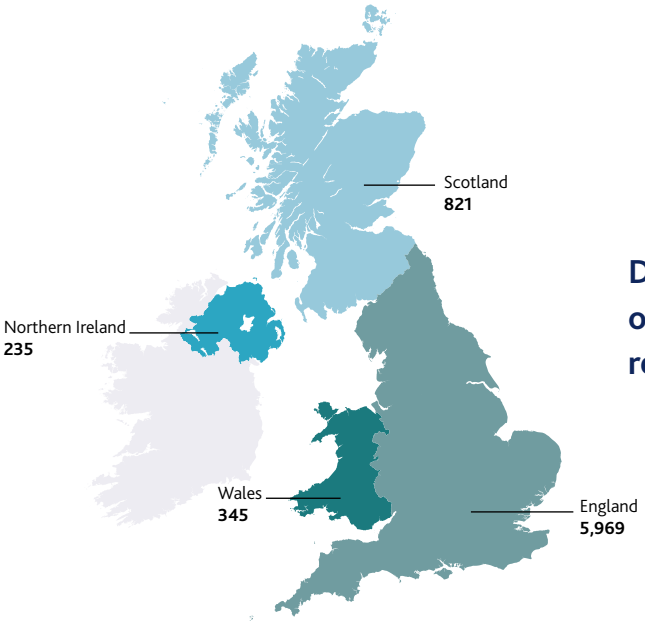
* The figure includes applications for full registration, provisional registration and temporary registration.

† Certificate of Eligibility for Specialist Registration (CESR) and a Certificate of Eligibility for GP Registration (CEGPR) are equivalence routes for doctors to gain this type of registration where they are not eligible for a Certificate of Completion of Training. You can find further details on these processes at www.gmc-uk.org/doctors/SpecApps.asp.

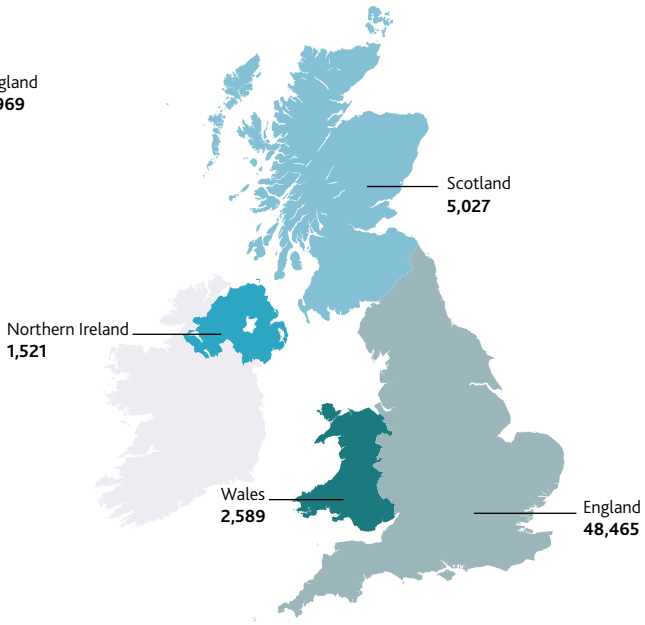
§ Of those 1,018 doctors who were granted registration but not a licence to practise in 2015, there were 282 who went on to re-apply for and be granted registration with a licence to practise later in 2015. The remaining 736 were not licensed and could not practise in the UK.

99% of applications were responded to within five working days

UK Medical School graduates of 2015 who gained provisional registration in 2015 by UK country of the medical school



Doctors revalidated in 2015 by country of the designated body submitting the recommendation*



* Number of doctors revalidated in 2015. The majority of these were a result of recommendations submitted by designated bodies, but a very small number were the result of internal decisions. These were classed against the country of the designated body they were connected to at the time.

The location is the location of the designated body that made the recommendation. For some, such as locums, this means it's the office of the locum agency rather than the location where the doctor was working.

Please note that this shows all decisions made in 2015 – the doctors may no longer be subject to revalidation.

9,418 concerns about doctors reviewed

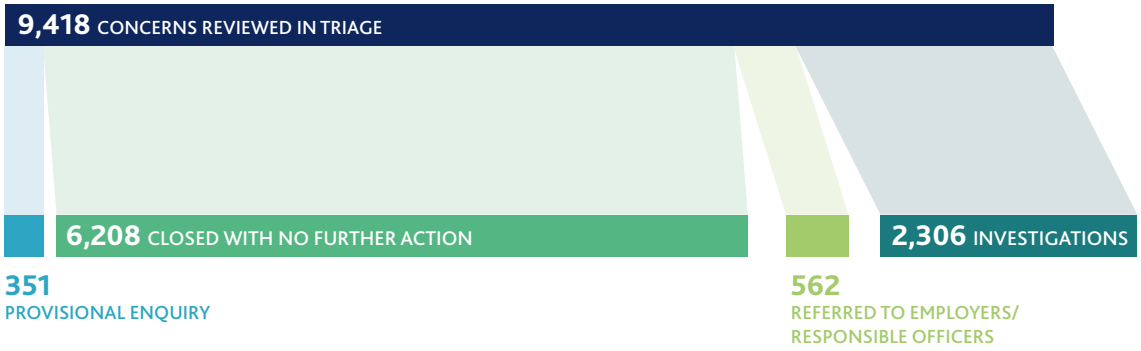
Taking action where concerns are raised

When a serious concern is raised about a doctor's behaviour, health or performance, we investigate to see if the doctor is putting the safety of patients, or the public's confidence in doctors, at risk.

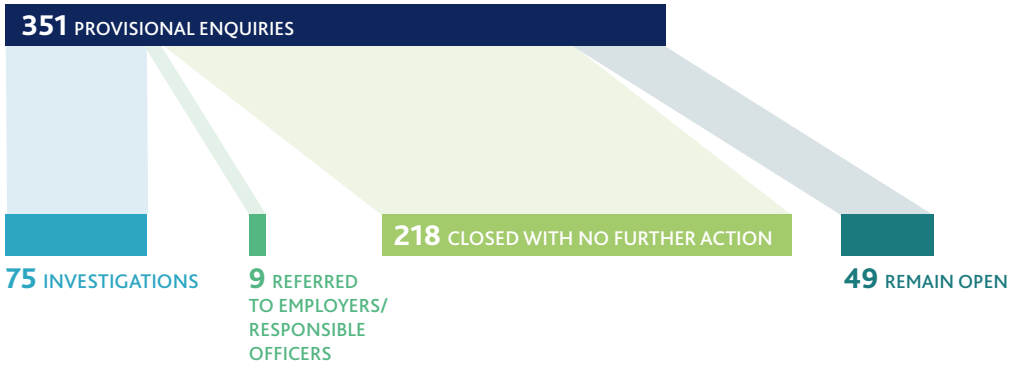
In 2015, we reviewed a total of 9,418 concerns about doctors. Of these, we opened investigations into 2,306 potentially serious concerns.

- We achieved our service target of concluding 90% of fitness to practise cases within 12 months, achieving 92%.
- We achieved 91% against our service target of concluding or referring 90% of cases at the investigation stage within six months.
- We achieved 95% against our service target of concluding or referring 95% of cases at the investigation stage within 12 months.
- We met our service target of reviewing 100% of doctors with conditions or undertakings attached to their registration before being returned to unrestricted registration.

2015 outcomes of triage



2015 outcomes of provisional enquiries



92% of fitness to practise cases concluded within 12 months

2015 outcomes of investigating concerns



* This figure shows the total outcomes from ongoing and new investigations in 2015 which have a case examiner decision. Because cases opened in 2015 will not necessarily reach an outcome in the same year, the figure does not track a single cohort of complaints. Therefore, the total number of investigations (2,566) is higher than the number of investigations from triage (2,381).

99% of fitness to practise tribunals started within nine months of referral

Independent adjudication of cases

The Medical Practitioners Tribunal Service (MPTS) is the adjudication service for UK doctors. It operates separately from the rest of our fitness to practise work,* so that all decisions are independent. In December 2015, this independence was strengthened with the MPTS becoming a statutory body. You can find more information about this important change on page 25.

When action is needed to protect patients or to maintain public confidence in doctors, an MPTS tribunal† can suspend a doctor's right to work, or restrict his or her practice – for example, by requiring them to work under supervision, or undergo further training. In the most serious cases, the MPTS tribunal can remove a doctor's name from the medical register.

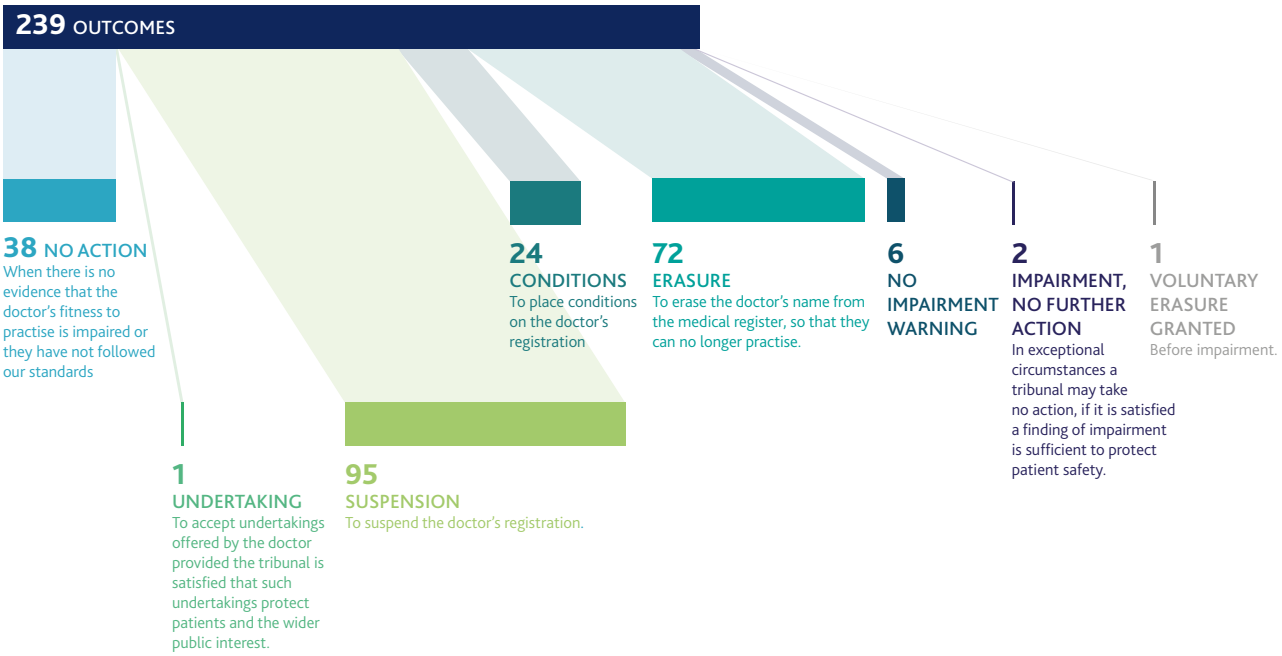
- The MPTS met its service target of starting 90% of fitness to practise tribunal hearings within nine months of referral, achieving 99%.
- The MPTS met its service target of starting 100% of interim orders tribunal hearings within three weeks of referral.

* See www.mpts-uk.org/about/1603.asp.

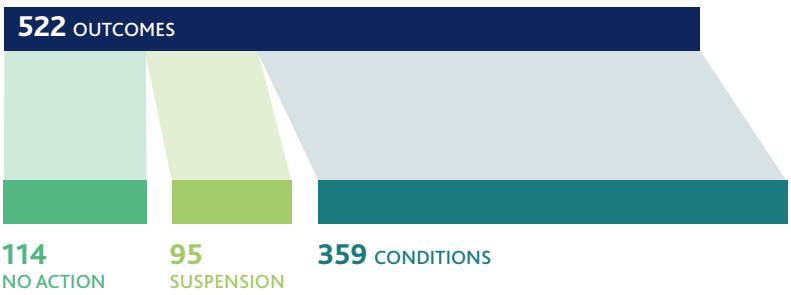
† In our annual report 2014 we referred to MPTS tribunals as 'MPTS panels' but the correct and ongoing naming convention is 'MPTS tribunals'. See www.gmc-uk.org/publications/corporate_publications.asp#Annualreport.

100% of interim orders tribunals started within three weeks of referral

2015 outcomes from MPTS fitness to practise tribunals



2015 outcomes from MPTS interim orders tribunals



25 enhanced monitoring issues resolved

Ensuring high-quality education and training for doctors

We set the educational standards for all UK doctors through undergraduate and postgraduate education and training. We promote high standards and make sure medical education and training reflect the needs of patients, medical students and doctors in training, and the healthcare systems across the UK. We also visit medical schools and training environments to check that our standards for education are being met.

Enhanced monitoring* is the process by which we support deaneries, local education and training boards and medical schools to manage concerns about quality and safety in medical education and training. In 2015, there were:

- 24 new enhanced monitoring issues created
- 25 enhanced monitoring issues resolved
- an average of 86 enhanced monitoring issues open at any time
- an average of three enhanced monitoring issues falling behind at any time.

Although the concerns are managed locally, we need frequent updates on them and we can provide additional support to help manage them where appropriate. If we are not satisfied that things are improving, we work with local organisations to facilitate the removal of trainees from environments that do not meet our standards.

95% of calls answered within 15 seconds

Responding to enquiries

Our contact centre is often the first contact point with our organisation for the public, employers, doctors and other regulators. We aim to provide excellent levels of service to the high volume of people who contact us. In 2015, our contact centre answered over 187,700 telephone calls and over 104,000 pieces of correspondence.

We increased our contact centre resourcing in 2014 to meet increasing demand and a change in the complexity of enquiries being handled by the team. In 2015, we achieved:

- 95% against our target of answering 90% of calls within 15 seconds
- 99% against our service target of answering 95% of emails and letters within five working days
- 97% against our target of seeing 95% of doctors within ten minutes of their arrival for an appointment
- 97% against our target of responding to 95% of complaints within ten working days.*

Information requests

In 2015, we received 435 subject access requests under the *Data Protection Act 1998*. This was a decrease of 18.8% from 2014. The number of information requests that we received under the *Freedom of Information Act 2000* in 2015 was 760. This was a 5.1% decrease from 2014.

- We achieved 85.5% against our target to respond to 80% of subject access requests within 40 days.
- We achieved 92.2% against our target to respond to 87.5% of freedom of information requests within 20 working days.

* Figure relates to complaints received by our Registration and Revalidation directorate.

2015 FINANCIAL REVIEW

The accounts for the year ended 31 December 2015 have been prepared in accordance with the new *Charities Statement of Recommended Practice (FRS 102)*, which replaces *UK Generally Accepted Accounting Practice* and the *Charities Statement of Recommended Practice 2005*. An explanation of the changes is set out in the notes to the accounts.

FRS 102 requires the 2014 accounts to be restated for comparative purposes. This restatement is shown in note 1 of the accounts and includes an accrual of £0.5 million for holiday entitlement arising in the year, which was due but not taken.

Our total income and expenditure for 2015

In 2015, we generated a total income of £99.3 million, and our operational expenditure was £101.2 million. Our income in 2015 increased by £3.5 million compared with 2014, largely due to an increase in the annual retention fee on 1 April 2015, coupled with a natural growth in the number of registered doctors. Our expenditure in 2015 reduced by £0.4 million compared with 2014; while we had budgeted for a higher level of expenditure, we were able to contain costs through proactive management.

We achieved £3.2 million of cashable efficiency gains in 2015, against a target of £2.6 million. These were generated through a range of initiatives including:

- carrying out more legal work in-house rather than using external advisers
- introducing paperless adjudication hearings and reducing the average hearing length
- deferring recruitment and reallocating work
- agreeing with HMRC a significant VAT saving on examination services
- a rent review on our office accommodation
- changes to our overtime and expenses policies
- reviewing our IT support contracts.

There were also fewer adjudication hearing days in 2015 than anticipated.

Each year, we set a business plan and budget based on our strategic aims and a forecast of likely work volumes. Our actual performance against our strategic aims is set out earlier in this report, and our financial performance against budget is summarised below.

Our income in 2015 was £0.4 million under budget. While the number of registered doctors increased over the year, the timing of doctors joining and leaving the medical register during the year differed from our budget assumptions. Also, our investment income was lower than budgeted. We originally planned to invest £50 million through fund managers but in the light of prevailing market conditions we took the decision to pause and review our investment approach. During the review we held a significant proportion of our cash in short-term and instant access accounts, for liquidity reasons, which typically generate lower returns than we had budgeted from the fund managers. We subsequently decided to reduce the sum invested through fund managers to £10 million, and deferred placing funds until 2016.

Our operational expenditure in 2015 was £2.9 million under budget.

Fitness to practise costs are a significant proportion of our total expenditure. Expenditure in 2015 was £0.5 million under budget. We had assumed that the pattern of increasing complaints about doctors, seen in recent years, would continue in 2015, but in the event volumes stabilised and so actual costs were lower than budgeted. Also, some recruitment was deferred, and more legal work was carried out in-house rather than using external lawyers. This in turn led to a reduction in adjudication hearings, so the costs incurred by the Medical Practitioners Tribunal Service (MPTS) were £0.5 million less than budgeted. We were also able to release accruals of £0.7 million as the actual level of fees and expenses claimed by associates in previous years has been lower than expected.

Our registration and revalidation costs were £0.7 million under budget, largely due to staff vacancies, reallocating work across teams and deferring recruitment.

Our strategy and communication costs were £0.2 million under budget because of staff vacancies during the year and project work being deferred.

By exercising tight control over our external consultancy requirements, costs were £0.2 million under budget. We have been steadily reducing our expenditure on external consultancy over recent years.

Depreciation charges were £0.1 million under budget, reflecting the nature and timing of our capital expenditure during the year.

Our education and standards, resources and accommodation costs were broadly in line with budget.

In 2015, we spent £6.3 million on major projects to improve our information systems infrastructure and accommodation.

Following a strong return on assets, our defined benefit pension scheme ended the year with a surplus of £21.5 million. This comprised assets of £165.2 million and liabilities of £143.7 million, valued in accordance with the financial reporting standard FRS 102. This is set out in more detail in note 16 of the accounts. The defined benefit pension scheme was closed to new joiners on 1 July 2013 and replaced by a defined contribution scheme.

Trustees' responsibilities for the financial statements

The accounts presented here have been prepared in accordance with the new *Charities Statement of Recommended Practice (FRS 102)*, which replaces *UK Generally Accepted Accounting Practice* and the *Charities Statement of Recommended Practice 2005*. Where necessary, the 2014 accounts have been restated for comparative purposes, and explanations for any changes are set out in the notes to the accounts.

The trustees are responsible for preparing the trustees' report and the financial statements in accordance with applicable law and regulations. Charity law says that the trustees must prepare financial statements for each financial year in accordance with the *Charities Statement of Recommended Practice (FRS 102)* and applicable law. Under charity law, the trustees must not approve the financial statements unless they are satisfied that they give a true and fair view of the state of affairs of the charity and of its net incoming resources for that period. In preparing these financial statements, the trustees have:

- selected suitable accounting policies and applied them consistently
- made judgements and estimates that are reasonable and prudent
- followed applicable accounting standards without any material departures
- prepared the financial statements on the going concern basis
- observed the methods and principles in the *Charities Statement of Recommended Practice (FRS 102)*.

The trustees are responsible for keeping adequate accounting records that are sufficient to show and explain the charity's transactions and disclose, with reasonable accuracy at any time, the financial position of the charity. The trustees are also responsible for making sure the financial statements comply with the *Charities Act 2011*, the *Charity (Accounts and Reports) Regulations 2008*, the provisions of the trust deed, the *Charities and Trustee Investment (Scotland) Act 2005*, the *Charities Accounts (Scotland) Regulations 2006* and the Privy Council Directions issued under the *Medical Act 1983*. The trustees are responsible for safeguarding the assets of the charity and for taking reasonable steps to prevent and detect fraud and other irregularities.

Related party transactions

International Standards on Auditing (ISAs) place significant emphasis on the identification, recording and disclosure of relationships and transactions with related parties. For ISA purposes, related parties are defined as close family members, members of the same household and business partners.

All trustees and senior managers must disclose details of any organisations in which they hold a position of authority or other material interest and whose business could bring them into financial contact with the GMC. They must also disclose details of any actual transactions between the GMC and related parties in the year.

In 2015, no such related party transactions were reported.

Reserves policy and going concern

Our level of reserves and our reserves policy are reviewed annually, and any financial implications are addressed as part of the budget-setting process.

We hold reserves:

- to fund working capital and manage the normal day-to-day cash flow of the business because our expenditure is broadly linear whereas income is concentrated in summer and winter peaks
- to provide funds to address the risks we have identified that may result in an unexpected increase in expenditure or a reduction in income
- to provide funds to respond to new initiatives and opportunities that come up during the year
- to fund the period between a decision to increase income and its taking full effect.

There is no standard formula that can be used to calculate the ideal level of reserves. We follow the Charity Commission's guidance and set a target range of reserves based on our cash flow requirements and an assessment of the risks facing the organisation. We aim to hold reserves at a level that is not excessive, but does not put our solvency at risk.

We operate a defined benefit pension scheme. In line with the accounting standard FRS 102, the value of the pension scheme assets and liabilities is recognised on the balance sheet. While the operation of the defined benefit pension scheme does create a financial risk for the organisation, any deficit or surplus in the scheme can be managed over the medium term, and so has no immediate impact on our cash flow requirements. Any risks associated with changes in the level of pension scheme assets and liabilities are therefore disregarded for reserves policy purposes.

A significant proportion of our total reserves is represented by fixed assets, which cannot easily be converted into cash at short notice without adversely affecting our ability to fulfil our charitable aims. The value of fixed assets is therefore disregarded for reserves policy purposes.

Based on our analysis of cash flows and the risks facing the organisation, our policy is to maintain free reserves in the range of £25 million–£45 million. But we recognise that the level of reserves will inevitably fluctuate year on year, reflecting variations in actual levels of income and expenditure compared with the budget. Our policy is to maintain actual free reserves in line with the target level over the medium term. If our actual reserves vary significantly from the target range set out in the reserves policy, we will address the variation as part of the annual budget-setting process to bring actual reserves back into line within a reasonable period.

Our free reserves at the end of 2014 stood at £36.3 million. As our costs in 2015 exceeded our income, our free reserves on 31 December 2015 reduced to £34.1 million. Total reserves at the end of the year were £69.4 million, made up of free reserves, plus £13.8 million of reserves represented by fixed assets, and a pension reserve of £21.5 million.

On 10 December 2015, we decided to embark on a major change programme to reduce our costs and increase our income over the medium term. This change programme involves additional costs in 2016 and 2017 to generate significant savings in future years. We also decided to increase the 2016–17 registration fees and annual retention fee in line with inflation, recognising the additional cost pressures we face. We estimate that our free reserves at the end of 2016 will reduce to around £30.9 million.

The majority of our income comes from registration fees paid by doctors. All doctors must be registered with us to practise medicine in the UK, and so our income is relatively certain. The trustees are therefore of the view that we are a going concern.

There are no material uncertainties related to events or conditions that cast significant doubt on our financial stability over the medium term.

Investment policy

In 2015, we reviewed our investment policy to improve yield while maintaining an appropriate degree of security and liquidity. Our previous policy was to hold general reserves in cash or near cash equivalents to minimise risk in terms of both loss of capital and volatility of investment returns. The new investment policy separates our funds into three categories:

- those that are needed as working capital for the normal day-to-day operation of the business
- those we invest under management
- the remaining cash balance, which fluctuates throughout the year.

We will hold £20 million as working capital, as cash in instant access interest-bearing accounts. We will invest £10 million under management starting in the early part of 2016, and the balance of our funds will be held in medium-term deposits.

For funds under management, we have a low-risk appetite and wish to protect against volatility, capital loss and the erosion of asset value by inflation. We aim to achieve a target rate of return on funds under management of the consumer price index plus 2% to 5%, over a rolling five-year period. We have appointed CCLA as our fund manager, investing through their COIF Ethical Fund.

We have adopted a comprehensive ethical investment approach. We seek to avoid those investments that we believe conflict with our charitable aims, and we may invest in companies whose activities are consistent with, or supportive of, our charitable aims. We may also use our position as an investor to influence the corporate behaviour of those companies we invest in.

Our policy is to invest only through fund managers who demonstrate the strongest environmental, social and governance credentials. We seek to specifically exclude investments in companies whose principal purpose is involvement with tobacco, alcohol, gambling, pornography, high-interest rate lending, cluster munitions and landmines.

Our Council is responsible for determining and reviewing the overall investment policy, objectives, risk appetite and target returns. Council has delegated to the Investment Sub-Committee responsibility for implementing the investment policy, appointing and managing fund managers, monitoring performance and reporting to Council.

Our 2015 accounts show cash needed for normal day-to-day working capital on our balance sheet within current assets, and cash held for the longer term is shown as investments.

In 2015, our investments generated interest of £0.5 million, equivalent to an average annual rate of return of 0.6%.

Key management personnel – pay and remuneration policy

The Remuneration Committee is responsible for determining the remuneration, benefits, and terms of service for the Chief Executive, Chief Operating Officer/Deputy Chief Executive, Senior Medical Adviser and GMC Responsible Officer, Chair of the MPTS and directors. The Committee sets all aspects of salary or honoraria, the provision of any other benefits, and any other arrangements or contractual terms for this group of staff.

The Committee considers that we should provide remuneration and rewards that will attract and retain the high-calibre staff necessary to enable us to fulfil our statutory remit and deliver our strategic objectives.

In setting the base pay for individual posts, the Committee will:

- take external advice on roles within its remit when new posts are established, posts undergo structural change and every two years for existing roles
- seek to align salaries with the upper quartile in the Croner reward consolidated database (using London salaries and the appropriate turnover comparators) or a suitable agreed alternative, subject to resource considerations

- consider all posts within its remit as UK-wide posts and not apply regional salary differentials based on location
- with regard to the Chair of the MPTS, continue to align salaries with that of senior circuit judges.

An annual consolidated pay award is considered with reference to the organisation's level of performance, the financial implications of any award, the award agreed for other GMC employees and wider market trends. An annual variable non-consolidated element is considered, reflecting personal performance, with regard to the same considerations applied to any consolidated award. The effectiveness of these arrangements is reviewed on an annual basis.

Staff within the Remuneration Committee's remit will usually be entitled to the benefits package available to all GMC employees on the same terms. The Committee retains the ability to withdraw, adjust or change any benefits for staff within its remit, subject to any consultation and contractual requirements. The Committee considers any additional benefits in kind (such as relocation payments) on a case-by-case basis.

New external staff appointees within the Committee's remit are automatically enrolled into our defined contribution scheme. Where employees have existing agreed pension arrangements, such as membership of the defined benefit scheme, they retain this for the course of their employment, subject to any changes to the rules agreed by trustees and the employer.

The Committee will make sure the equality and diversity implications of remuneration policy and related decisions are considered appropriately. Specifically, it will make sure:

- any salary differentials are supported by a formal job evaluation or independent external market advice
- any decisions relating to variable pay are supported by an objective assessment of performance
- any adjustment or changes to remuneration arrangements do not discriminate unlawfully
- other decisions relating to terms of service are supported by appropriate advice on any equality and diversity implications.

Audit and Risk Committee's report

The Audit and Risk Committee has eight members – six Council members and two external members – providing relevant experience and knowledge of finance, governance and risk. The Committee continues to be a key part of our governance structure providing Council with independent assurance on the effectiveness of arrangements to ensure the:

- integrity of the financial statements
- effectiveness of the systems of internal control, governance and risk management
- adequacy of both the internal and external audit services.

The Committee bases its advice and decisions on guidance issued by the Financial Reporting Council and, where appropriate, independent external advice.

In 2015, the Committee held four meetings and submitted two formal reports on its work and findings to our Council. Key activities during the year were:

- approving the external audit letter of engagement and reviewing the outcome of the external auditor's work
- participating in the procurement tender exercise to appoint a new external audit provider for three years beginning in January 2016 and making an appointment recommendation to Council
- overseeing delivery of year one of the 2015–17 internal audit programme, scrutinising all audit findings to satisfy itself that the actions proposed are appropriate and monitoring the implementation of recommendations to make sure they are being managed effectively by the Executive
- challenging the Corporate Risk Register at every meeting and scrutinising specific risks in seminar sessions
- overseeing an evaluation of the risk management framework to provide assurance that risk management is evolving with business maturity and is in line with the principles of effective risk management set out in the international guidance standard ISO31000:2009.

Risk management statement

Our approach to risk management is set out in the risk management framework. Both Council and the Audit and Risk Committee have discussed risks regularly throughout the year. The executive Performance and Resources Board regularly monitors existing corporate and emerging risks and mitigating actions. We capture and monitor local operational risks through directorate and project risk registers.

In 2015 an independent review of our risk management concluded that we have an established and appropriately-tailored risk management framework in place, which is embedded and active across the organisation.

Understanding and responding to patient safety lie at the core of our organisational purpose. We continue to maintain robust systems and procedures to mitigate the risk of failure to deliver our statutory functions, which are at the heart of protecting patients and promoting good medical practice throughout a doctor's training and professional career. This includes, for example, attention to the areas where individuals:

- seek to fraudulently gain access to the medical register
- provide misleading information in a fitness to practise case
- don't comply with the requirements of our standards for education, training and curricula
- fail to comply with revalidation requirements to demonstrate they remain fit to practise.

We are also alert to the inherent internal risks of being an employer of over 1,000 staff. Our rigorous recruitment procedures, induction and ongoing training and development provide strong mitigating controls. In particular, given the volume and complexity of data handling on a day-to-day basis, we give regular information security training to staff and we remain certified to the Information Security Management international standard ISO 27001.

Horizon scanning and being alert to emerging operational and strategic risks are part of ongoing business oversight by the executive board. Operational emerging risks we addressed in 2015 include:

- being compliant with *Directive 2005/36/EC* the *Recognition of Professional Qualifications European Directive* by the January 2016 deadline
- preparing for the implementation of an organisational change programme without destabilising business-as-usual activities
- improving the use of the data and information we hold internally and across the broader health service to support patient safety.

Our strategic risks are largely driven by the external environment, which means we have limited control of the actions that may need to be taken. Nevertheless, we play an important role in bringing such issues to the attention of others and working collaboratively where possible to drive appropriate actions and outcomes. For example, in 2015, we produced joint guidance on the duty of candour with the Nursing and Midwifery Council and we continued to monitor and discuss with partners:

- the impact of potential changes in European legislation
- legislative reform proposals in the UK
- government proposals to move the point of registration for doctors joining the medical register
- the implementation of the *Welsh Language Standards Regulations 2015*.

We also work collaboratively with the system regulators and improvement bodies in Scotland, Wales, Northern Ireland and England, to share information where we have identified potential patient safety concerns that relate to the healthcare environment rather than an individual doctor.

But alongside threats, risk also brings opportunities. Over the past 12 months we've been able to take some important steps.

- The implementation of the DH Section 60 order has allowed us to enhance the MPTS and elements of fitness to practise activities.
- The introduction of a database that helps us to better understand a doctor's progression through their career and the environments they work. We've also increased our work on promoting professionalism and improving awareness of our role. And we've created more opportunities for doctors to engage with our professional standards – such as through our Welcome to UK Practice initiative, which helps doctors new to practice or new to the country understand the ethical issues that will affect them and their patients on a day-to-day basis.
- Publishing more information about doctors on our online register, such as doctors in training and their training specialty, identifying doctors who are approved trainers and more information about a doctor's revalidation.

Like other bodies in the healthcare sector, we continue to face significant external environment challenges such as:

- public sector financial pressures continue to place strain on the health service and the individuals who serve and use it
- the junior doctors' industrial action has an impact on patients and registrants' future careers, and has longer-term consequences for the health service in general
- proposals and timing of potential regulatory reform are unclear
- the impact of elections in Scotland, Wales and Northern Ireland and the potential implications for healthcare regulations
- uncertainty over whether we will leave or remain in the European Union.

But there are opportunities too.

- We have an opportunity to develop a Medical Licensing Assessment, which will further support consistency of standards of medical practice for those who seek to join the medical register.
- We are exploring how to better use our capabilities and intellectual property to the benefit of others around the world as well as providing a potential income stream for us.
- We are embarking on a programme of internal transformation, which will enhance the effectiveness and efficiency of business operations and make sure we have a sustainable financial base for the future.
- We continue to work with regulatory and healthcare partners to find areas of collaboration with the aim of raising standards of care, improving patient safety and driving efficiency across the regulatory sector.

We will continue to be vigilant in monitoring both current and emerging risks ensuring we remain alert and active in taking appropriate mitigating actions and using opportunities to work with partners where we can to continue the drive to improving medical standards of practice and patient safety.

OUR STRUCTURE,
GOVERNANCE AND MANAGEMENT

Council and other governance groups

Council is our governing body. It comprises 12 members, six of whom are medical members and six of whom are lay members. Our Council members are also the trustees of the organisation. Council contains members from the four countries of the UK.

The trustees between 1 January 2015 and 31 December 2015 were:

- Dr Shree Datta, MBBS BSc (Hons) MRCOG LLM
- Lady Christine Eames, OBE LLB MPhil
- Professor Michael Farthing, MD DSc(Med) FRCP FMedSci
- Rt. Hon. Baroness Hayman, GBE MA PC
- Rt. Hon. Professor The Lord Kakkar, BSc (Hons) MBBS (Hons) Phd FRCS PC
- Professor Deirdre Kelly, CBE MD FRCP FRCPI FRCPCH
- Dame Suzi Leather, DBE MBE MA BA BPhil CQSW LLD (Hons) FRCOG (Hons) FRSH (Hons) DL
- Mr Julian Lee, FCA
- Professor Jim McKillop, BSc MB ChB PhD FRCP FRCR
- Dame Denise Platt, DBE CBE BSc Econ
- Mrs Enid Rowlands, BSc CCMI
- Professor Terence Stephenson, BSc (Hons) DM FRCPCH FRCP FRACP (Hons) FRCPI (Hons) FRCS (Hons) FHKAP (Hons) FRCGP (Hons) FRCA (Hons) FCAI (Hons) FRCS Edin (Hons) FRCOG (Hons) FAcadMEd (Hons) FRCP Edin (Hons).

The trustees were all independently appointed by the Privy Council, through a process that followed the Professional Standards Authority's guidance for making appointments to healthcare professional regulatory bodies.

Two new members were appointed in 2015: Professor Terence Stephenson was appointed Chair of Council, and Mr Julian Lee was appointed as a Council member. Both were appointed for a four-year term with effect from 1 January 2015.

An induction programme was carried out to make sure that the Council members had the information they needed to support them, including briefings on operational areas of work (for example, trusteeship, our legislative framework and on the public sector Equality Duty).

Council examined its progress in taking forward a review of its effectiveness in 2015, including:

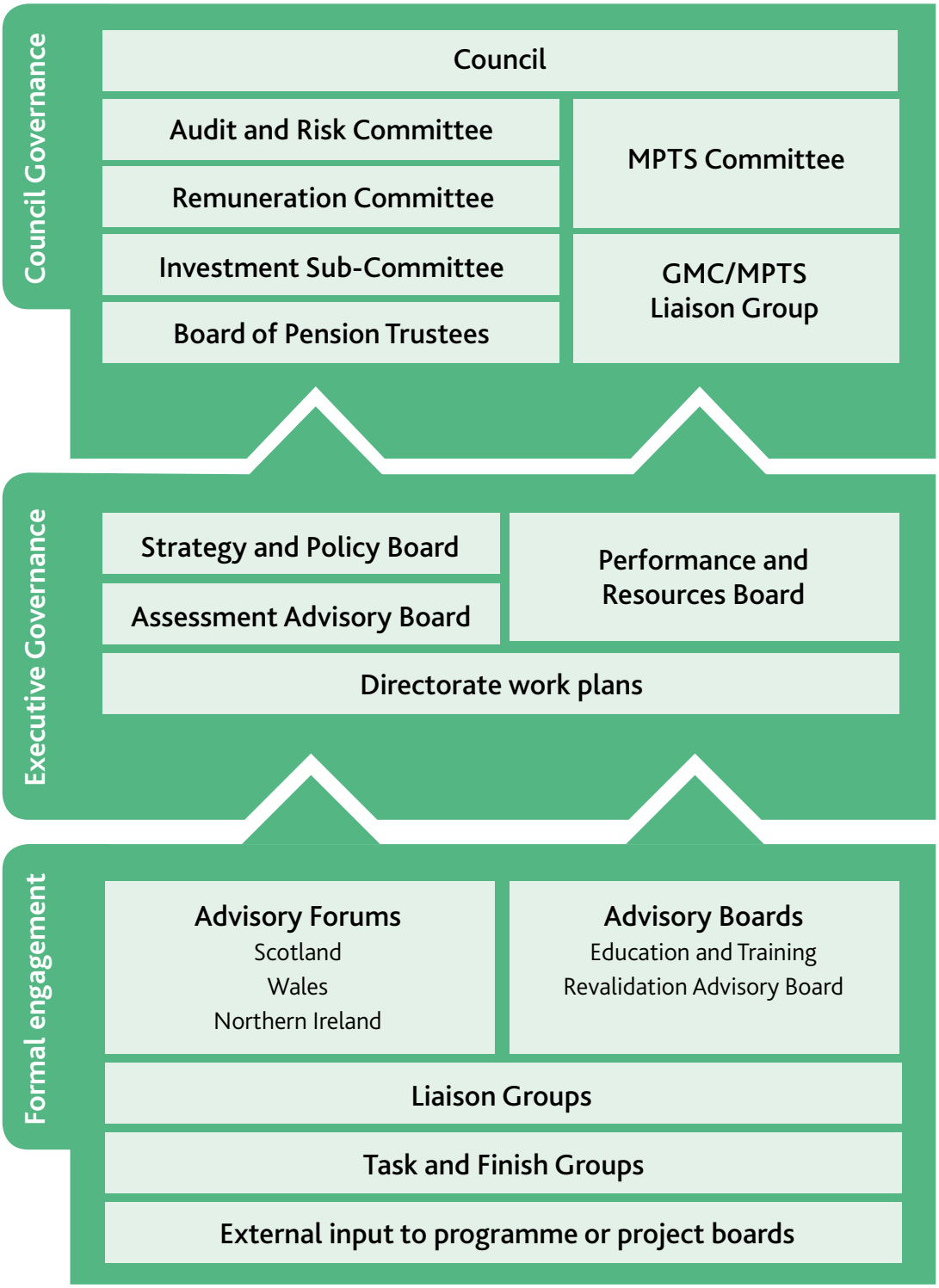
- reviewing its priorities to make sure its forward work programme reflected issues that needed strategic input
- agreeing revised performance reporting in relation to its strategic priorities and operational key performance indicators
- implementing changes to its working arrangements to hold more seminar and informal sessions to enhance its knowledge of operations and engagement with staff
- overseeing the development of a communications and engagement strategy.

Council members participated in appraisal reviews in 2015, which included consideration of any learning and development needs.

Council reviewed the arrangements for the Council member appointments and reappointments process, which takes place in 2016, to fill the vacancies from two members (Professor Jim McKillop and Mrs Enid Rowlands) demitting office and for the eight members who are eligible for reappointment having reached the end of their first term of office.

We carried out a skills audit at the end of 2015 to inform planning for Council's future needs and to identify the skills and experience that we may need in the new appointments and reappointments. An appointments campaign to fill the vacancies will begin in June 2016, under the same independent process used for existing Council members.

The diagram on the next page shows the different governance groups that assist Council in discharging their responsibilities. These have all been agreed by Council to help them oversee our work effectively.


Council Governance
Council
Audit and Risk Committee
Remuneration Committee
Investment Sub-Committee
Board of Pension Trustees
MPTS Committee
GMC/MPTS Liaison Group
Executive Governance
Strategy and Policy Board
Assessment Advisory Board
Performance and Resources Board
Directorate work plans
Formal engagement
Advisory Forums
Scotland
Wales
Northern Ireland
Advisory Boards
Education and Training
Revalidation Advisory Board
Liaison Groups
Task and Finish Groups
External input to programme or project boards

Audit and Risk Committee

You can find the Audit and Risk Committee's report on page 60. Since 1 January 2015, the Audit and Risk Committee has been chaired by Professor Jim McKillop.

Remuneration Committee

The Remuneration Committee is chaired by Dame Denise Platt. It advises Council on the remuneration, the terms of service and the expenses policy for Council members, including the Chair. It also determines the appointment process for the Chief Executive and MPTS Chair and the remuneration, benefits, and terms of service for the Chief Executive, Chief Operating Officer/Deputy Chief Executive, directors, and MPTS Chair and MPTS Committee members. It also has responsibility for ensuring that the assessment and measurement of performance, and the assessment of recruitment and succession planning, take place within an appropriate framework for the senior management roles within its remit. The Committee reports annually to Council.

Dr Shree Datta was appointed as a member of the Committee from 1 January 2015.

Investment Sub-Committee

The Investment Sub-Committee is chaired by Dame Suzi Leather. It has two external co-opted members, with recruitment to fill a vacancy being carried out in the first half of 2016. It is responsible for implementing and reviewing our investment policy, making sure the management of assets is consistent with the investment policy, appointing and managing fund managers, and monitoring performance. The Sub-Committee reports on investment performance to Council through the Chief Operating Officer's report, as well as reporting annually on its activities to Council.

Board of Pension Trustees

Our Staff Superannuation Scheme is managed and administered by a Board of Trustees and chaired by Lord Kirkwood of Kirkhope, in accordance with the scheme's Trust Deed and Rules. Mrs Enid Rowlands stood down from the board at the end of 2015 and Professor Deirdre Kelly was appointed as a new employer-nominated member from 1 January 2016.

Medical Practitioners Tribunal Service

The MPTS has responsibility for overseeing the adjudication of fitness to practise hearings, and is led by the Chair of the MPTS, His Honour David Pearl.

The MPTS Committee (formerly known as the MPTS Advisory Committee) and joint GMC and MPTS Liaison Group continue as part of the governance framework. The GMC and MPTS Liaison Group oversees the working relationship between the MPTS and the functions of the GMC with which it interacts. Since January 2015, the GMC and MPTS Liaison Group has been chaired by Professor Terence Stephenson, Chair of Council. The MPTS Committee is chaired by His Honour David Pearl.

In March 2015, changes to the *Medical Act 1983* established the MPTS on a statutory basis. They gave us the power to appeal decisions of MPTS tribunals when it considers a tribunal decision does not sufficiently protect the public, a power that didn't previously exist. The rule changes to establish the MPTS as a statutory committee, accountable directly to Parliament, came into force on 31 December 2015. Work was carried out in 2015 to review the Governance handbook and GMC/MPTS Operational Framework, and changes were made to reflect the new statutory and non-statutory working arrangements and operational procedures ready for implementation in 2016.

Recruitment campaigns will take place in 2016 to appoint a new MPTS Chair, to fill the vacancy when His Honour David Pearl stands down in early 2017, and to appoint a new MPTS Committee member to fill a vacancy following one of the members standing down.

Strategy and Policy Board

Our Strategy and Policy Board is an executive board, and is chaired by our Chief Executive, Niall Dickson. It includes members of our Senior Management Team. The board is an advisory forum for Niall Dickson, which gives advice and recommendations on areas including:

- supporting Council in strategy development
- policy development priorities and significant changes to existing policy, including information and research to support strategy and policy development
- linkages between policy development and legislation
- external engagement in the organisation's strategy and policy development.

The board reports its work through the Chief Executive's reports and an annual report to Council.

Performance and Resources Board

The Performance and Resources Board is an executive board, chaired by our Chief Operating Officer Susan Goldsmith. It includes members of our Senior Management Team.

The board is an advisory forum for the Chief Operating Officer, and gives advice and recommendations to the Chief Executive on areas including:

- business and operational planning
- performance management and reporting
- resource management
- risk management and related controls
- quality assurance, efficiency and continuous improvement
- making sure that equality and diversity are integrated into our core activities, including monitoring action plans and compliance with the equality duty.

The board also oversees the advisory Management Committee, which monitors and reviews our staff Defined Contribution Pension Scheme. The board reports its work through the Chief Operating Officer's reports and an annual report to Council.

The board will also oversee delivery of the change programme in 2016. You can find more information on the change programme on page 39.

UK advisory forums

We have established advisory forums in Scotland, Wales and Northern Ireland, which are chaired by Professor Terence Stephenson, Chair of Council. The forums support Council's role in making sure we have effective engagement with interest groups, and that our policies are suited to the context in all parts of the UK.

The forums are in addition to our existing arrangements for engagement and are intended to give a structured setting for us to engage on medium- and long-term priorities, and to share and discuss any early-stage views on policy development. The forums report their work to the Strategy and Policy Board.

Education and Training Advisory Board

The Education and Training Advisory Board is chaired by Professor John Connell. It gives us advice on the delivery of undergraduate and postgraduate medical education and training, and career progression.

The board's advice is crucial in developing our policy and in making sure that Council is fully briefed before major decisions are made. The board's invited membership reflects the diverse range of those who have an interest in medical education and training across the UK. The board reports its work to the Strategy and Policy Board.

Revalidation Advisory Board

The Revalidation Advisory Board is chaired by Sir Keith Pearson. It gives us advice about how effectively revalidation has been operating since it was introduced in December 2012.

The board gives insight from a range of perspectives about how the system is working on the ground, and how different groups, including doctors, Responsible Officers, patients, the public and employers, are experiencing revalidation. It is an important part of how we monitor implementation, as well as issues relating to the evaluation of revalidation and whether revalidation is being delivered as envisaged. The board reports its work to the Strategy and Policy Board.

Assessment Advisory Board

The Assessment Advisory Board was established in 2015 under the leadership of Professor Val Wass. It gives us expert advice on the development and operation of GMC-led assessments and assessments that we oversee.

A range of experts in assessment and assessment design, including those with expertise in differential attainment and statistical analysis of examination performance (psychometrics), and a lay member have joined the board. Its work is reported to the Strategy and Policy Board.

Member attendance at Council, boards and committees in 2015

Member and trustee	Number of meetings attended
Dr Shree Datta	
Council	8/8
Remuneration Committee	2/2
Investment Sub-Committee	4/5
Lady Christine Eames	
Council	8/8
Audit and Risk Committee	4/4
UK Advisory Forums – Northern Ireland*	2/2
Professor Michael Farthing	
Council	6/8
Audit and Risk Committee	4/4
Baroness Helene Hayman	
Council	8/8
Remuneration Committee	2/2
Professor The Lord Ajay Kakkar	
Council	6/8
Remuneration Committee	1/2
Investment Sub-Committee	4/5
Professor Deirdre Kelly	
Council	7/8
Audit and Risk Committee	3/4
Dame Suzi Leather	
Council	7/8
Audit and Risk Committee	2/4
Investment Sub-Committee	5/5

* Council member attendance at the forum meetings is on a voluntary basis on the invitation of the Chair of Council.

Member and trustee	Number of meetings attended
Mr Julian Lee	
Council	7/8
Audit and Risk Committee	4/4
Investment Sub-Committee	5/5
Professor Jim McKillop	
Council	8/8
Audit and Risk Committee	4/4
Board of Trustees of the GMC's Superannuation Scheme	4/4
UK Advisory Forums – Scotland*	2/2
Dame Denise Platt	
Council	8/8
Remuneration Committee	2/2
Mrs Enid Rowlands	
Council	8/8
Remuneration Committee	2/2
Board of Trustees of the GMC's Superannuation Scheme	4/4
UK Advisory Forums – Wales*	2/2
Professor Terence Stephenson	
Council	8/8
Remuneration Committee	2/2
GMC and MPTS Liaison Group	2/2
UK Advisory Forums – Northern Ireland	2/2
UK Advisory Forums – Scotland	2/2
UK Advisory Forums – Wales	2/2

* Council member attendance at the forum meetings is on a voluntary basis on the invitation of the Chair of Council.

Management

In 2015, our staff were under the direction of Chief Executive Niall Dickson and Chief Operating Officer and Deputy Chief Executive Susan Goldsmith.

On 31 December 2015, the directors were:

- Paul Buckley
  Director of Strategy and Communication
- Vicky Osgood
  Director of Education and Standards
- Judith Hulf
  Interim Director of Education and Standards and Senior Medical Adviser and Responsible Officer
- Una Lane
  Director of Registration and Revalidation
- Anthony Omo
  General Counsel and Director of Fitness to Practise
- Neil Roberts
  Director of Resources and Quality Assurance.

In January 2016, Niall Dickson announced that he would step down as Chief Executive and Registrar at the end of 2016, after seven years in the role. Our Chair and Council will oversee the recruitment of a new Chief Executive.

Our principal places of business are 3 Hardman Street, Manchester M3 3AW and Regent's Place, 350 Euston Road, London NW1 3JN. We also have offices in Belfast, Cardiff and Edinburgh and a centre for hearings, where the MPTS is based, at St James's Buildings, 79 Oxford Street, Manchester M1 6FQ.

Professional advisers

Bankers

Royal Bank of Scotland
250 Bishopsgate
London
EC2M 4AA

Solicitors

The majority of our legal work is carried out by our in-house legal team.

Auditors

Crowe Clark Whitehill LLP
St Bride's House
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Approved by the trustees on 7 June 2016, and signed on their behalf by:

Professor Terence Stephenson
Chair of Council

Independent auditors' report to the trustees of the General Medical Council

We have audited the financial statements of General Medical Council for the year ended 31 December 2015 which comprise the Statement of Financial Activities, the Balance Sheet, the Cash Flow Statement and the related notes numbered 1 to 18.

The financial reporting framework that has been applied in their preparation is applicable law and FRS 102, The Financial Reporting Standard applicable in the UK and Republic of Ireland.

This report is made solely to the charity's trustees, as a body, in accordance with section 154 of the *Charities Act 2011* and section 44(1c) of the Charities *and Trustee Investment (Scotland) Act 2005*. Our audit work has been undertaken so that we might state to the charity's trustees those matters we are required to state to them in an auditor's report and for no other purpose. To the fullest extent permitted by law, we do not accept or assume responsibility to anyone other than the charity and the charity's trustees as a body, for our audit work, for this report, or for the opinions we have formed.

Respective responsibilities of trustees and auditors

As explained more fully in the Statement of Trustees' Responsibilities, the trustees are responsible for the preparation of the financial statements and for being satisfied that they give a true and fair view.

We have been appointed as auditor under section 144 of the *Charities Act 2011* and section 44(1c) of the *Charities and Trustee Investment (Scotland) Act 2005* and report in accordance with those Acts.

Our responsibility is to audit and express an opinion on the financial statements in accordance with applicable law and International Standards on Auditing (UK and Ireland). Those standards require us to comply with the Auditing Practices Board's Ethical Standards for Auditors.

Scope of the audit of the financial statements

An audit involves obtaining evidence about the amounts and disclosures in the financial statements sufficient to give reasonable assurance that the financial statements are free from material misstatement, whether caused by fraud or error. This includes an assessment of:

- whether the accounting policies are appropriate to the charity's circumstances and have been consistently applied and adequately disclosed;
- the reasonableness of significant accounting estimates made by the trustees; and
- the overall presentation of the financial statements.

In addition, we read all the financial and non-financial information in the Trustees' Annual Report and any other surround information to identify material inconsistencies with the audited financial statements and to identify any information that is apparently materially incorrect based on, or materially inconsistent with, the knowledge acquired by us in the course of performing the audit. If we become aware of any apparent material misstatements or inconsistencies we consider the implications for our report.

Opinion on financial statements

In our opinion the financial statements:

- give a true and fair view of the state of the charity's affairs as at 31 December 2015 and of its incoming resources and application of resources, for the year then ended;
- have been properly prepared in accordance with United Kingdom Generally Accepted Accounting Practice; and
- have been prepared in accordance with the requirements of the *Charities Act 2011* and the *Charities and Trustee Investment (Scotland) Act 2005* and Regulation 8 of the *Charities Accounts (Scotland) Regulations 2006* and the *Medical Act 1983* and the Privy Council Directions issued thereunder.

Matters on which we are required to report by exception

We have nothing to report in respect of the following matters where the *Charities Act 2011* or the *Charities Accounts (Scotland) Regulations 2006* (as amended) requires us to report to you if, in our opinion:

- the information given in the Trustees' Annual Report is inconsistent in any material respect with the financial statements
- sufficient accounting records have not been kept; or
- the financial statements are not in agreement with the accounting records and returns; or
- we have not received all the information and explanations we require for our audit.

Crowe Clark Whitehill LLP
Statutory Auditor
London

Crowe Clark Whitehill LLP is eligible to act as an auditor in terms of section 1212 of the *Companies Act 2006*.

ACCOUNTS **2015**

Statement of financial activities for the year ended 31 December 2015

	Note	Total 2015 £000	Total 2014 (restated) £000
Income from			
Charitable activities			
Registration	2	93,832	90,735
Specialist and GP registration	2	3,557	3,270
Other trading activities	3	492	537
Investments	3	1,264	1,029
Other	3	116	195
Total income		**99,261**	95,766
Expenditure on			
Charitable activities			
Fitness to practise		49,901	49,309
Registration and revalidation		20,304	20,879
Medical Practitioners Tribunal Service		13,333	14,434
Education		7,412	7,315
External relationships		5,983	5,559
Communications		2,895	2,721
Standards		1,367	1,362
Total expenditure	**5**	**101,195**	101,579
Net income/(expenditure)		**(1,934)**	(5,813)
Other recognised gains/(losses)			
Actuarial gain/(loss) on defined benefit pension scheme	16	3,670	8,596
Net movement in funds		**1,736**	2,783
Total funds brought forward		67,670	64,887
Total funds carried forward		**69,406**	67,670

Balance sheet as at 31 December 2015

	Note	2015 £000	2015 £000	2014 (restated) £000	2014 (restated) £000
Fixed assets					
Tangible fixed assets	7		6,900		8,440
Intangible fixed assets	8		6,917		5,777
Investments	9		60,000		60,000
Total fixed assets			**73,817**		**74,217**
Current assets					
Debtors and prepayments	10	19,862		18,019	
Cash and bank balances		26,039		91	
Short-term deposits		-		25,242	
Total current assets		**45,901**		**43,352**	
Liabilities					
Creditors: amounts falling due within one year	11	(68,410)		(66,031)	
Net current liabilities			**(22,509)**		**(22,679)**
Total assets less current liabilities			**51,308**		**51,538**
Provisions for liabilities and charges	12		(3,406)		(991)
Net assets excluding pension scheme asset			**47,902**		**50,547**
Defined benefit pension scheme asset	16		21,504		17,123
Total net assets			**69,406**		**67,670**
The funds of the charity					
Unrestricted income funds			47,902		50,547
Pension reserve			21,504		17,123
Total charity funds	**13**		**69,406**		**67,670**

The financial statements were approved by the trustees and authorised for issue on 7 June 2016'.
They were signed on behalf of the trustees by:

Professor Terence Stephenson
Chair of Council

Cash flow statement for the year ended 31 December 2015

	2015		2014 (restated)	
	£000'	£000'	£000'	£000'
Cash flows from operating activities:				
Net cash provided by/(used in) operating activities (note 1 below)		**6,514**		**3,641**
Cash flows from investing activities:				
Dividends, interest and rents from investments	512		643	
Purchase of property, plant and equipment	(6,320)		(7,804)	
Net cash provided by/(used in) investing activities		**(5,808)**		**(7,161)**
Net cash provided by/(used in) financing activities		-		-
Change in cash and cash equivalents (note 2 below)		**706**		**(3,520)**

Note 1

Reconciliation of net income/(expenditure) to net cash flow from operating activities	£000'	£000'
Net income/(expenditure) for the reporting period (as per the statement of financial activities)	(1,934)	(5,813)
Adjustments for:		
Investment income and interest	(1,264)	(1,029)
Non-cash items – depreciation	6,720	6,834
Non-cash items – assets written off	-	10
Pension past service cost and curtailment	-	36
Pension scheme current service cost	9,049	8,565
Pension scheme contribution	(9,008)	(7,420)
(Increase)/decrease in debtors	1,843	(85)
Increase/(decrease) in creditors and provisions	4,794	2,543
Net cash provided by (used in) operating activities	**6,514**	**3,641**

Note 2 Cash and equivalents	Short-term deposits £000'	Cash at bank and in hand £000'	Total £000'
Balances at 1 January 2015	25,242	91	25,333
Net increase/(decrease) in cash and cash equivalents	(25,242)	25,948	706
Balances at 31 December 2015	**-**	**26,039**	**26,039**

Notes to the accounts

1. Principal accounting policies

Accounting convention

Our financial statements have been prepared on a going concern basis and in accordance with the *Charities Statement of Recommended Practice (FRS 102)*, applicable to charities preparing their accounts in accordance with the Financial Reporting Standard applicable in the UK and Republic of Ireland, the *Charities Act 2011*, the *Charities and Trustee Investment (Scotland) Act 2005*, the *Charities Accounts (Scotland) Regulations 2006* and UK Generally Accepted Accounting Practice as it applies from 1 January 2015. The GMC meets the definition of a 'public benefit entity' under FRS 102.

The accounts have been prepared to give a 'true and fair' view and have departed from the *Charities (Accounts and Reports) Regulations 2008* only to the extent required to provide a 'true and fair' view. This departure has involved following Accounting and Reporting by Charities preparing their accounts in accordance with the Financial Reporting Standard applicable in the UK and Republic of Ireland (FRS 102) issued on 16 July 2014 rather than the *Accounting and Reporting by Charities: Statement of Recommended Practice* effective from 1 April 2005, which has since been withdrawn.

In preparing the accounts, the trustees have considered whether in applying the accounting policies required by FRS 102 and the *Charities SORP (FRS 102)* a restatement of comparative items was required. The statement on the next page shows a restatement of the 2014 accounts as if they had been prepared under FRS 102 for comparative purposes with 2015. At the date of transition in applying the requirement to recognise liabilities arising from employee benefits, a liability was recognised for holiday entitlement carried forward and for the entitlement arising in the year, which was due but not taken. The initial liability was for £555,000. No other restatement impacts on the opening balances.

	Total 2014 £000'	Movement	Restated 2014 £000'
Income			
From charitable activities			
Registration	90,735	–	90,735
Specialist and GP registration	3,270	–	3,270
Other trading activities	537	–	537
Investments	2,317	(1,288)	1,029
Other	195		195
Total income	**97,054**	**(1,288)**	**95,766**
Expenditure			
Charitable activities			
Fitness to practise	45,658	3,651	49,309
MPTS	13,767	667	14,434
Registration and revalidation	18,809	2,070	20,879
Education	6,732	583	7,315
Standards	1,249	113	1,362
Communications	2,542	179	2,721
External relationships	5,153	406	5,559
Governance	7,114	(7,114)	–
Total expenditure	**101,024**	**555**	**101,579**
Net income/(expenditure) before investments	**(3,970)**	**(1,843)**	**(5,813)**
Net gain/(losses) on investment	**–**	**–**	**–**
Net income/(expenditure) before recognised gains and losses	**(3,970)**	**(1,843)**	**(5,813)**

	Total 2014 £000'	Movement	Restated 2014 £000'
Other recognised gains/(losses) on investments			
Actuarial gain/(loss) on defined benefit pension scheme	7,308	1,288	8,596
Net movement in funds	**3,338**	**(555)**	**2,783**
Total funds brought forward	64,887	–	64,887
Total funds carried forward	**68,225**	**(555)**	**67,670**

Expenditure	2014	% allocation	Holiday pay provision	Governance	Restated
Fitness to practise	45,658	47.61%	264	3,387	49,309
MPTS	13,767	8.71%	48	619	14,434
Registration and revalidation	18,809	26.99%	150	1,920	20,879
Education	6,732	7.61%	42	541	7,315
Standards	1,249	1.47%	8	105	1,362
Communications	2,542	2.33%	13	166	2,721
External relationships	5,153	5.28%	30	376	5,559
Governance	7,114	(100.00%)	–	(7,114)	–
Total expenditure	**101,024**	–	**555**	–	**101,579**

Reconciliation of net income/(expenditure) to net cash flow from operating activities	2014	Pension	Holiday pay provision	Restated
Net income/(expenditure)	(3,970)	(1,288)	(555)	(5,813)
Investment income and interest	(2,317)	1,288	–	(1,029)
Non-cash items – depreciation	6,834	–	–	6,834
Non-cash items – assets written off	10	–	–	10
Pension past service cost and curtailment	36	–	–	36
Pension scheme current service cost	8,565	–	–	8,565
Pension scheme contribution	(7,420)	–	–	(7,420)
(Increase)/decrease in debtors	(85)	–	–	(85)
Increase/(decrease) in creditors	1,988	–	555	2,543
Net cash provided by/(used in) operating activities	**3,641**	**–**	**–**	**3,641**

The principal accounting policies adopted in the preparation of the financial statements, which have been applied consistently, are detailed below.

Income
Income is included in the statement of financial activities when all of the following criteria are met:

- Entitlement – control over the rights or other access to the economic benefit has passed to the GMC.
- Probability – it is more likely than not that the economic benefits will flow to the GMC.
- Measurement – the value can be measured reliably.

The following specific policies apply.

- Annual retention fees relate to services to be provided over a 12-month period. Income is deferred and released to the statement of financial activities on a straight-line basis over the period to which the income relates.
- Registration fees, including provisional registration fees, are recognised when registration is granted.
- Professional and Linguistic Assessments Board (PLAB) fees are recognised when the examinations are sat.
- Income from investments and funds held on deposit is recognised when it is receivable and the amount can be accurately measured.

  All income is recognised gross.

Basis for recognising liabilities
Expenditure includes staffing costs, office costs, committee costs, legal costs, accommodation costs, purchase of assets, and financial, actuarial and professional costs.

Expenditure is included in the statement of financial activities on an accruals basis. All liabilities are recognised as soon as there is a legal or constructive obligation committing the charity to expenditure.

Basis for allocation of expenditure
The majority of our resources are expended directly in pursuit of our charitable aims, and are identified as such in the statement of financial activities.

Accommodation costs, governance costs and other support costs are apportioned to charitable activities on the basis of staff head count across the organisation.

Irrecoverable VAT
Any irrecoverable VAT is charged to the statement of financial activities as part of the relevant item of expenditure, or capitalised as part of the cost of the related asset where appropriate.

Taxation
We can take advantage of the exemptions from taxation on income and gains available to charities, so no taxation is payable on the net incoming resources.

Debtors
Trade and other debtors are normally recognised at the settlement amount due after any trade discount offered. Prepayments are normally valued at the amount prepaid net of any trade discounts due.

Creditors and provisions for liabilities
Creditors and provisions are recognised when the charity has a present legal or constructive obligation as a result of a past event. They are recognised when it is probable that a transfer of economic benefit will be required to settle the obligation and a reliable estimate can be made of the amount of the obligation. Creditors and provisions are normally recognised at their settlement amount after allowing for any trade discounts due.

Critical accounting judgements and key sources of estimation uncertainty
The key sources of estimation uncertainty that have a significant effect on the amounts recognised in the financial statements are:

- All unsettled claims for legal costs made against the GMC are reviewed on a case-by-case basis at the year end. Provisions are based on historical experience and a detailed assessment of the specific details of current cases. The final settlement of cases is dependent on a number of factors, so the accuracy of the provision is subject to a significant degree of uncertainty.
- Provisions for property dilapidation costs are assessed on a case-by-case basis, close to the lease end date when a reasonable estimate of costs can be made.
- Provisions for holiday pay are based on the actual level of accrued days and salaries of each staff member.

Tangible fixed assets
Tangible fixed assets are stated at cost, net of depreciation and any provision for impairment. Expenditure is only capitalised where the cost of the asset or group of assets acquired exceeds £5,000.

Intangible fixed assets
Intangible fixed assets comprise computer software. They are stated at cost, net of depreciation and any provision for impairment. Expenditure is only capitalised where the cost of the asset or group of assets acquired exceeds £5,000.

Depreciation
Depreciation is provided so as to write off the cost, less estimated residual value, of the assets evenly over their estimated useful lives. In the case of leased assets, the cost is written off over the period of the lease. The period of the lease is determined as the period up to the first break clause, unless our intention is not to exercise the break.

The estimated useful lives are as follows.

- Leasehold buildings and leasehold improvements – the period of the lease or the useful economic life of the asset.
- Furniture, fixtures, and office fittings – the lesser of either five years or the remaining term of the lease.
- Information Technology (IT) equipment and software – three years.
- Other office equipment – three years for IT-related items and five years for all other items.
- Depreciation rates are reviewed on a regular basis comparing actual lives of assets with the accounting policy rates.

Operating leases
Rent payable under operating leases is charged to the statement of financial activities on a straight-line basis over the period of the lease.

Financial instruments
The charity has financial assets and liabilities of a kind that qualify as basic financial instruments. Basic financial instruments are initially recognised at transaction value and subsequently measured at amortised cost. Financial assets held at amortised cost consist of cash and bank balances, short-term deposits, investments held in cash deposits together with trade and other debtors. Financial liabilities held at amortised cost comprise trade and other creditors, tax and social security creditors and accruals.

Investments and cash
Our investment policy separates our funds into three categories: those which are required as working capital for the normal day-to-day operation of the business; those which we invest under management; and the remaining cash balance which fluctuates during the year.

Funds held as cash for the normal day-to-day operation of the business are shown on the GMC's balance sheet within current assets, while funds held for the longer term are shown as investments.

Pensions
We have a defined benefit pension scheme for permanent employees. The scheme was closed to new members on 30 June 2013, and replaced by a defined contribution scheme. The surplus or deficit of the defined benefit scheme is recognised on the balance sheet. Changes in the assets and liabilities of the scheme are disclosed and allocated as follows:

- Charges relating to current or past service costs, and gains and losses on settlements and curtailments, are included within staff costs and charged to the statement of financial activities.
- Interest on the net defined benefit asset/liability is shown as a net amount of other finance costs or as an incoming resource alongside investment income and interest. Actuarial gains and losses are recognised immediately in other recognised gains and losses on investments.
- The assets, liabilities and movements in the surplus or deficit of the scheme are calculated by qualified independent actuaries as an update to the latest full actuarial valuation. Details of the defined benefit scheme assets, liabilities and major assumptions are shown in the notes to the accounts.
- Our defined contribution pension scheme was set up on 1 July 2013. Contributions to the scheme are charged to the statement of financial activities in the year in which they are payable to the scheme.
- A small number of staff who transferred to the GMC on the merger with the Postgraduate Medical Education and Training Board (PMETB) contribute to the NHS multi-employer scheme and contributions to the scheme are charged to the statement of financial activities in the year in which they are payable to the scheme.

Funds and reserves
All of our funds are unrestricted, and can be expended at the trustees' discretion, in pursuit of our charitable aims.

2. Income from charitable activities

	Total 2015 £000'	Total 2014 £000'
Registration		
Annual retention fees	88,561	85,455
Registration fees	2,931	3,291
Provisional registration fees	709	697
PLAB fees	1,495	1,157
Other fees	136	135
	93,832	**90,735**
Specialist and GP registration*		
Certificates of Completion of Training fees	2,583	2,507
Certificate of Eligibility for Specialist Registration/Certificate of Eligibility for General Practitioner Registration fees	938	746
Other fees	**36**	**17**
	3,557	**3,270**

* Specialist and GP registration was called Certification in the 2014 accounts.

3. Income from generated funds

	2015		2014	
	£000'	£000'	£000'	£000'
Activities for generating funds				
Other trading activities†	492		537	
Other	116	608	195	732
Investment income				
Other finance income – pension scheme (note 16)	752		386	
Bank interest	512	1,264	643	1,029
		1,872		**2,493**

† Other trading activities include sales of the medical register, external hire of the Clinical Assessment Centre, the reimbursement of costs of visiting overseas medical schools and the reimbursement of costs of staff seconded to external bodies.

4. Financial Instruments

	2015	2014
	£000'	£000'
Financial assets measured at amortised cost	86,469	85,855
Financial liabilities measured at amortised cost	14,261	15,891
The entity's income, expense, gains and losses in respect of financial instruments are summarised below:		
Total interest income for financial assets held at amortised cost	**512**	**643**

5. Total expenditure

	Direct staffing costs	Direct costs	Allocated costs	Total 2015	Total 2014
	£000'	£000'	£000'	£000'	£000'
Fitness to practise	21,310	11,308	17,283	**49,901**	49,309
Registration and revalidation	8,710	2,684	8,910	**20,304**	20,879
Medical Practitioners Tribunal Service	3,205	7,068	3,060	**13,333**	14,434
Education	4,259	452	2,701	**7,412**	7,315
External relationships*	3,344	613	2,026	**5,983**	5,559
Communications	1,570	333	992	**2,895**	2,721
Standards	791	23	553	**1,367**	1,362
Total charitable expenditure	**43,189**	**22,481**	**35,525**	**101,195**	**101,579**

* External relationships include the work done by our Regional Liaison Service, strategic relationships, our offices in Northern Ireland, Scotland and Wales, and our European and international development activities.

	Support costs allocated to charitable activities								
	Management	IT	Human resources	Finance	Procurement	Facilities	Governance	Total 2015	Total 2014
	£000'	£000'	£000'	£000'	£000'	£000'	£000'	£000'	£000'
Fitness to practise	1,316	5,179	2,264	509	202	4,812	3,001	**17,283**	16,604
Registration and revalidation	679	2,670	1,167	262	104	2,481	1,547	**8,910**	9,415
Medical Practitioners Tribunal Service	233	917	401	89	36	852	532	**3,060**	3,037
Education	206	809	354	79	32	752	469	**2,701**	2,653
External relationships	154	607	265	60	24	564	352	**2,026**	1,875
Communications	76	297	130	29	12	276	172	**992**	749
Standards	42	166	73	16	6	154	96	**553**	515
Total charitable expenditure	**2,706**	**10,645**	**4,654**	**1,044**	**416**	**9,891**	**6,169**	**35,525**	**34,848**

Support costs and governance costs are managed centrally, and then allocated to charitable activities on the basis of staff head count across the organisation.

	2015	2014
	£000'	£000'
Staffing costs	58,310	54,944
Office costs	3,458	4,302
Council and committee costs	411	512
Tribunal and assessment costs	14,801	15,950
Legal costs	4,902	6,106
Accommodation costs	6,717	6,493
Financial, actuarial and professional costs	3,830	4,346
Purchase of assets – charged to revenue	2,046	2,083
Assets written off	–	9
Depreciation	6,720	6,834
	101,195	**101,579**

Total expenditure includes:

Operating lease costs: leasehold property and equipment	3,389	3,467
Audit fees	41	40

6. Staff

	2015 £000'	2014 £000'
Total costs of all staff		
Salaries	42,165	40,038
Social security costs	3,458	3,090
Superannuation costs – defined benefit scheme	6,187	7,255
Superannuation costs – defined contribution scheme	1,299	645
Redundancy costs	2,286	192
Other staffing costs	2,915	3,724
	58,310	**54,944**

	2015	2014
Average staff numbers (fulltime equivalents) in the year by category		
Fitness to practise	423	345
Medical Practitioners Tribunal Service	75	71
Registration and revalidation	218	200
Standards	14	11
Education	66	61
Communications	24	19
External relations	50	41
Governance	51	55
Resources	161	142
	1,082	**945**

The number of staff whose taxable emoluments (excluding redundancy payments) fell into higher salary bands was:

	2015	2014
GMC		
£60,000–£70,000	32	27
£70,001–£80,000	27	30
£80,001–£90,000	16	17
£90,001–£100,000	11	6
£100,001–£110,000	8	5
£110,001–£120,000	4	5
£120,001–£130,000	3	2
£130,001–£140,000	1	1
£140,001–£150,000	1	1
£150,001–£160,000	–	–
£160,001–£170,000	1	–
£170,001–£180,000	–	1
£180,001–£190,000	4	4
£190,001–£200,000	2	–
£200,001–£210,000	–	–
£210,001–£220,000	–	–
£220,001–£230,000	–	–
£230,001–£240,000	1	1

	2015	2014
MPTS		
£70,001–£80,000	1	1
£80,001–£90,000	–	–
£90,001–£100,000	1	1
£100,001–£110,000	–	–
£110,001–£120,000	1	1

	2015	2014
Number of staff included above for whom retirement benefits are accruing		
GMC defined benefit pension scheme	95	94
GMC defined contribution pension scheme	13	3
NHS defined benefit pension scheme	1	3
Not in scheme	2	3
	111	**103**

The key management personnel of the charity include the Chief Executive, Chief Operating Officer and all directors. The total employee benefits of key management personnel were £1,533,743 (2014 £1,328,201). The Chief Operating Officer post was vacant for a significant part of 2014.

7. Tangible fixed assets

	Buildings	Fixtures, furniture and equipment	IT equipment	Total
	£000'	£000'	£000'	£000'
Cost				
Balance at 1 January 2015	5,665	7,859	8,711	22,235
Additions	-	803	1,042	1,845
Disposals	-	(114)	(563)	(677)
Balance at 31 December 2015	**5,665**	**8,548**	**9,190**	**23,403**
Depreciation				
Balance at 1 January 2015	5,623	3,851	4,321	13,795
Depreciation charge for year	23	1,310	2,052	3,385
Disposals	–	(114)	(563)	(677)
Balance at 31 December 2015	**5,646**	**5,047**	**5,810**	**16,503**
Net book value at 1 January 2015	42	4,008	4,390	8,440
Net book value at 31 December 2015	19	3,501	3,380	6,900

As part of the transition to FRS 102 we have taken the opportunity to review our classification of fixed assets between buildings, fixtures and IT equipment. Computer system software is now shown separately as intangible fixed assets in note 8.

8. Intangible fixed assets

	Computer software and systems development £000'
Cost	
Balance at 1 January 2015	25,504
Additions	4,475
Disposals	(261)
Balance at 31 December 2015	**29,718**
Amortisation	
Balance at 1 January 2015	19,727
Amortisation charge for year	3,335
Disposals	(261)
Balance at 31 December 2015	**22,801**
Net book value at 1 January 2015	5,777
Net book value at 31 December 2015	**6,917**

9. Investments

	2015 £000'	2014 £000'
The valuation at the end of the year consisted of:		
Cash deposits	**60,000**	60,000

10. Debtors

	2015	2014
	£000'	£000'
Amounts falling due within one year		
Registration debtors	16,591	15,390
Prepayments and accrued income	2,841	2,107
Other debtors	430	522
	19,862	**18,019**

11. Creditors

	2015	2014
	£000'	£000'
Amounts falling due within one year		
Trade creditors	941	720
Other creditors	1,151	1,034
Tax and social security	14	5
Holiday pay	581	555
Accruals	12,155	14,132
Deferred income	53,568	49,585
	68,410	**66,031**

Income from annual retention fees is deferred and released to the statement of financial activities on a straight-line basis over the period to which the income relates. All deferred income brought forward from the previous year is automatically released to the statement of financial activities in the following year.

	Annual retention fee	PLAB fees	Specialist and GP registration fees	Total
	£000'	£000'	£000'	£000'
Deferred income at 1 January 2015	49,083	345	157	**49,585**
Resources deferred during the year	53,076	477	15	**53,568**
Amounts released from previous years	(49,083)	(345)	(157)	**(49,585)**
Deferred income at 31 December 2015	**53,076**	**477**	**15**	**53,568**

12. Provisions

	2015	2014 (restated)
	£000'	£000'
Dilapidations	509	511
Legal claims	397	480
Change programme	2,500	–
	3,406	**991**

Dilapidations – each year we review our property leases and make provision for dilapidations, where the cost can be reasonably estimated.

Legal claims – each year the GMC makes a provision for potential costs related to ongoing legal cases.

Change programme – on 10 December 2015 the GMC decided to embark on a major change programme to reduce costs and increase income over the medium term. A provision was created in 2015 for restructuring costs associated with this change programme.

13. Fund movements in the year

	Unrestricted funds	Pension fund	2015 total	2014 total
	£000'	£000'	£000'	£000'
At 1 January 2015	50,547	17,123	67,670	64,887
Net incoming/(outgoing) resources	(2,645)	4,381	1,736	2,783
At 31 December 2015	**47,902**	**21,504**	**69,406**	**67,670**

14. Capital commitments

Capital expenditure contracted but unspent at 31 December 2015 amounted to £269,928.
The equivalent figure for 2014 was £274,852.

15. Operating lease commitments

	Land and buildings		Equipment	
	2015	2014	2015	2014
	£000'	£000'	£000'	£000'
Expiry date				
Within one year	4,333	4,262	57	114
In years two to five	16,758	16,623	-	57
After more than five years	13,599	17,682	-	-
	34,690	**38,567**	**57**	**171**

16. Superannuation schemes

The GMC has three staff pension schemes:

GMC Group Personal Pension Plan
This is a defined contribution pension scheme, which was set up on 1 July 2013. We started auto enrolment on 1 November 2013. At the end of 2015 there were 441 members of staff contributing to this scheme. It meets the UK Government's requirements following the introduction of automatic enrolment. Individuals can choose to make additional contributions by deduction from salary to the scheme. Under the terms of FRS 102, contributions are accounted for as a defined contribution scheme based on actual contributions paid through the year.

NHS Multi-Employer Scheme
We have five members of staff who contribute to the NHS multi-employer scheme, which is a defined benefit scheme. These staff transferred to the GMC on the merger with PMETB. The scheme operates as a pooled arrangement, with contributions paid at a centrally agreed rate. The trustees are unable to confirm the GMC's share of the underlying assets and liabilities of the NHS scheme and so, under the terms of FRS 102, contributions are accounted for as if the scheme were a defined contribution scheme based on actual contributions paid through the year.

GMC Staff Superannuation Scheme
This is a funded scheme of the defined benefit type, providing retirement benefits based on final salary. The top-up arrangement is an unfunded scheme.

This scheme was closed to new members on 30 June 2013, and replaced by the GMC Group Personal Pension Plan. At the end of 2015, there were 624 members of staff contributing to this scheme.

The FRS 102 valuation has been based on a full assessment of the liabilities for the scheme as at 31 December 2012. The present values of the defined benefit obligation, the related current service cost and any past service costs were measured using the projected unit credit method.

Actuarial gains and losses have been recognised in the period in which they occur (but outside the profit and loss account) through the Other Comprehensive Income.

Regular employer contributions to the scheme in 2016 are estimated to be £7,572,000.

The responsibility for investing pension scheme assets rests with pension trustees. *The Pensions Act 1995* requires trustees to draw up a Statement of Investment Principles, setting out the scheme's investment strategy. Pension trustees are required to consult the employer (GMC) when drawing up the strategy, but do not require the employer's formal agreement. Following consultation with the GMC, in 2014 the pension trustees adopted a fiduciary management approach to the investment of the scheme's assets.

The principal assumptions used by the independent qualified actuaries to calculate the liabilities under FRS 102 are set out below.

Main financial assumptions

	31 December 2015	31 December 2014	31 December 2013
	% pa	% pa	% pa
Retail Prices Index inflation	3.4	3.3	3.7
Consumer Price Index inflation	2.5	2.4	3.1
Rate of general long-term increase in salaries	4.9	4.8	5.2
Pension increases (excess over guaranteed minimum pension)	2.5	2.4	3.1
Discount rate for scheme liabilities	3.8	3.6	4.4

Mortality assumptions

The mortality assumptions are based on standard mortality tables which allow for expected future mortality improvements. The assumptions are that a member currently aged 65 will live on average for a further 23.7 years if they are male and for a further 25.8 years if they are female.

For a member who retires in 2035 at age 65 the assumptions are that they will live on average for a further 25 years after retirement if they are male and for a further 27 years after retirement if they are female.

Scheme asset allocation

	31 December 2015		31 December 2014	
	£000'	%	£000'	%
Delegated Consulting Service	162,329	98%	149,029	98%
Other	2,821	2%	3,032	2%
Total	**165,150**	**100%**	**152,061**	**100%**

The Delegated Consulting Service (DCS) is a fiduciary management solution that invests in a wide range of underlying assets in order to meet the scheme's specific investment objectives. The underlying asset allocation changes over time, based on the views of the fiduciary manager within the overall bounds set by the trustees. Under this approach the majority of scheme assets are invested in pooled funds. The managers of the pooled funds are required to have in place a policy on social, environmental and ethical considerations. The DCS fund includes a value of £32,602,000 in respect of hedge funds, based on November month end prices.

None of the scheme assets are invested in the organisation's financial instruments or in property occupied by, or other assets used by, the GMC.

Reconciliation of funded status to balance sheet

	31 December 2015	31 December 2014	31 December 2013
	£000'	£000'	£000'
Fair value of assets	165,150	152,061	125,491
Present value of funded defined benefit obligations	(142,670)	(134,255)	(115,489)
Funded status	22,480	17,806	10,002
Present value of unfunded defined benefit obligation	(976)	(683)	(680)
Asset/(liability) recognised on the balance sheet	**21,504**	**17,123**	**9,322**

Amounts recognised in income statement

	Year ending 31 December 2015	Year ending 31 December 2014
Operating cost:	£000'	£000'
Current service cost	9,049	8,565
Past service cost	-	36
Financing cost:		
Interest on net defined benefit liability/(asset)	(752)	(386)
Pension expense recognised in profit and loss	**8,297**	**8,215**

Amounts recognised in Other Comprehensive Income

	Year ending 31 December 2015	Year ending 31 December 2014
	£000'	£000'
Asset gains/(losses) arising during the year	224	14,311
Liability gains/(losses) arising during the year	3,446	(5,715)
Pension expense recognised in profit and loss	**3,670**	**8,596**

Changes to the present value of the defined benefit obligation during the year

	Year ending 31 December 2015 £000'	Year ending 31 December 2014 £000'
Opening defined benefit obligation (DBO)	134,938	116,169
Current service cost	9,049	8,565
Interest expense on DBO	4,826	5,279
Actuarial (gains)/losses on liabilities	(3,446)	5,715
Net benefits paid out	(1,721)	(826)
Past service cost	–	36
Closing defined benefit obligation	**143,646**	**134,938**

Changes to the fair value of scheme assets during the year

	Year ending 31 December 2015 £000'	Year ending 31 December 2014 £000'
Opening fair value of scheme assets	152,061	125,491
Interest income on scheme assets	5,578	5,665
Gain/(loss) on scheme assets	224	14,311
Contributions by the Company	9,008	7,420
Net benefits paid out	(1,721)	(826)
Closing fair value of scheme assets	**165,150**	**152,061**

Actual return on scheme assets

	Year ending 31 December 2015 £000'	Year ending 31 December 2014 £000'
Interest income on scheme assets	5,578	5,665
Gain/(loss) on scheme assets	224	14,311
Return on scheme assets	**5,802**	**19,976**

17. Honoraria

Trustees	2015 £	2014 £
Professor Sir Peter Rubin	–	110,000
Professor Terence Stephenson (Chair)*	110,000	–
Dr Shree Datta	18,000	18,000
Lady Christine Eames	18,000	18,000
Professor Michael Farthing	18,000	18,000
Baroness Helene Hayman	18,000	18,000
Professor The Lord Ajay Kakkar	18,000	18,000
Professor Deirdre Kelly	18,000	18,000
Dame Suzi Leather	18,000	18,000
Professor Jim McKillop	18,000	18,000
Dame Denise Platt	18,000	18,000
Mrs Enid Rowlands	18,000	18,000
Dr Hamish Wilson	-	18,000
Mr Julian Lee	18,000	-

* Honorarium paid to employer.

Medical Practitioners Tribunal Service committee members	2015 £	2014 £
His Honour David Pearl†	–	–
Mr Richard Davies	3,720	3,720
Dr Tim Howard	3,720	3,720
Dr Patricia Moultrie	3,720	3,720
Mrs Judith Worthington	3,720	3,720

† His Honour David Pearl is the Chair of the MPTS and is paid as an employee. His remuneration is included in note 5 of these accounts.

Audit and Risk Committee co-opted members	2015 £	2014 £
Ms Elizabeth Butler	1,395	2,480
Mr John Morley	1,240	1,550

Investment Sub-Committee co-opted members	2015 £	2014 £
Mr Tim Scholefield	2,480	–
Mr Jason Britton	2,480	–

18. Travel and subsistence expenses claimed in 2015

Trustees	2015 £	2014 £
Professor Sir Peter Rubin	–	8,890
Professor Terence Stephenson (Chair)	12,008	–
Dr Shree Datta	808	563
Lady Christine Eames	5,689	5,014
Professor Michael Farthing	113	122
Baroness Helene Hayman	265	165
Professor The Lord Ajay Kakkar	–	–
Professor Deirdre Kelly	1,657	996
Dame Suzi Leather	4,153	3,115
Professor Jim McKillop	5,937	4,330
Dame Denise Platt	610	466
Mrs Enid Rowlands	311	1,992
Dr Hamish Wilson	–	5,852
Mr Julian Lee	2,469	–
Total	**34,020**	**31,505**

Medical Practitioners Tribunal Service committee members	2015 £	2014 £
His Honour David Pearl	679	1,754
Mr Richard Davies	543	563
Dr Tim Howard	890	1,944
Dr Patricia Moultrie	382	1,027
Mrs Judith Worthington	581	858
Audit and Risk Committee co-opted members		
Ms Elizabeth Butler	37	347
Mr John Morley	14	6
Investment Sub-Committee co-opted members		
Mr Tim Scholefield	182	–
Mr Jason Britton	936	–

Senior Management Team	2015 £	2014 £
Niall Dickson – Chief Executive	29,061	12,200
Susan Goldsmith – Chief Operating Officer and Deputy Chief Executive *	6,109	2,963
Paul Philip – Chief Operating Officer and Deputy Chief Executive *	–	151
Paul Buckley – Director of Strategy and Communication	2,805	6,418
Judith Hulf – Director of Education and Standards †	2,675	2,257
Vicky Osgood – Director of Education and Standards †	4,814	–
Neil Roberts – Director of Resources and Quality Assurance	14,002	8,624
Una Lane – Director of Registration and Revalidation	8,768	5,617
Anthony Omo – General Counsel and Director of Fitness to Practise	9,884	13,365

* Susan Goldsmith joined the GMC on 6 October 2014 following Paul Philip's departure on 31 January 2014.
† Judith Hulf is acting Director of Education and Standards to cover Vicky Osgood's absence.

Variations in expenses reflect that the trustees, committee members and the Senior Management Team live in different parts of the UK and are required to travel around the UK on GMC business, including to our offices in London, Manchester, Edinburgh, Belfast and Cardiff, and occasionally outside the UK.

Adjustments are also made for those with disabilities, which may mean that additional expenses are incurred for travel and accommodation according to specific needs.

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